

Chapter 16 simulator row — 1

#	Where	Old text	New text	Source / reason
1	Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework fo Step 4b · raw	Score a state with strong Policy and Economics but weak Operations. The composite collapse is the argument for this chapter in one number.	Score a state with strong Policy and Economics but weak Operations. The composite barely moves — Operations is last in the sequence, so nothing waits behind it. What weak Operations costs is execution, not score: that gap is this chapter’s argument.	Phase 1, step 4b (2026-10-07). 1. EAST Fund "\$138M per year / annually" — WRONG. Primary source: GMCB, "AHEAD" December 2024 Board Meeting deck (12.6.24), slide 34: "VT receives a portion of these savings via a Medicare payment 'bump' (estimated \$138.9M increase in 2026)" = increase in Vermont's Medicare FFS TCOC target in PY1; 2-3% of participating hospitals' Medicare FFS NPR to care transformation, remainder to the EAST Fund. Vermont Public 2026-07-28 (Sec. Samuelson): "an extra \$138 million ... capped additional payments at \$10 million." A first-performance-year estimate, not an annual EAST Fund amount. 2. Ch16 "Work This Chapter" HTR Simulator row promised a composite "collapse" for weak Operations. The engine (lib/framework/sequence-engine.ts) implements Chapter 1's model: Operations is last in the build sequence, so nothing waits behind it (Ops 60→10 moves the composite 65.6→57.8). The book yields to Chapter 1: describe what the reader actually sees and why.

Preface / Introduction wording — 3

#	Where	Old text	New text	Source / reason
2	PREFACE Step 4a · raw	“Explore on the Platform” guide that maps the chapter’s argument to the specific tools that let you manipulate it, and a “Go Deeper — Academy”	“Work This Chapter on the Platform”	Phase 1, step 4a — book errors surfaced by the promise-delivery audit (2026-10-07). Preface: chapters end with 'Work This Chapter on the Platform' (15 sections); no 'Explore on the Platform' or 'Go Deeper — Academy' sections exist (only 1 of 15 links the Academy). Name the real section; point to the companion course (Introduction: Five Pillars, One Imperative follows the book’s sequence). Appendix F.6: AHEAD participants are MD, CT, HI, RI, downstate NY; Vermont withdrew July 2026 (/ahead-model page).
3	PREFACE Step 4a · raw	“Explore on the Platform”	“Work This Chapter on the Platform”	Phase 1, step 4a — book errors surfaced by the promise-delivery audit (2026-10-07). Preface: chapters end with 'Work This Chapter on the Platform' (15 sections); no 'Explore on the Platform' or 'Go Deeper — Academy' sections exist (only 1 of 15 links the Academy). Name the real section; point to the companion course (Introduction: Five Pillars, One Imperative follows the book’s sequence). Appendix F.6: AHEAD participants are MD, CT, HI, RI, downstate NY; Vermont withdrew July 2026 (/ahead-model page).
4	PREFACE Step 5 · raw	Vermont now ranks fourth-oldest in the country by median age (42.8), behind Maine (45.1), New Hampshire (43.0), and West Virginia (42.9).	Vermont now ranks second-oldest in the country by median age (43.6), tied with New Hampshire and behind only Maine (44.8); West Virginia (43.0) is next.	#ERROR!

UVMMC (court claim) — 5

#	Where	Old text	New text	Source / reason
5	INTRODUCTION Step 1 · raw	UVMMC’s subsequent court challenge to that enforcement — Vermont’s largest, most powerful hospital system testing GMCB’s authority directly — was decided against UVMMC.	UVMMC’s subsequent appeal of that enforcement — Vermont’s largest, most powerful hospital system testing GMCB’s authority directly — ended in April 2025, when UVMMC dismissed its appeals under a settlement that left the enforcement order in place.	— Part A ————— 1. UVMMC — Introduction

6	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 1 · raw	UVMMC challenged GMCB enforcement in court and lost. Future challenges are certain.	UVMMC appealed GMCB’s enforcement, then dismissed its appeals in an April 2025 settlement that left the enforcement order standing. Future challenges are certain.	1. UVMMC — Chapter 2 ("If you are a Vermont legislator")
7	Conclusion — What Vermont Proves, and What Remains to Be Proven Step 1 · raw	UVMMC challenged GMCB enforcement in court and lost. Vermont hospitals have contested	UVMMC challenged GMCB enforcement in court, then dropped its appeals in an April 2025 settlement that left the enforcement order in place. Vermont hospitals have contested	#ERROR!
8	Appendix G — Adapting and Stress-Testing the Framework Step 1 · raw	UVMMC challenged that enforcement in court. The challenge was decided against UVMMC. Three things follow from this. First, the corrective-action and budget-order mechanisms GMCB used are now judicially tested, not merely statutory — a materially stronger form of “mandatory” than untested authority. Second, the outcome did not turn on some narrow technicality that leaves the broader question open; GMCB’s core enforcement authority was upheld. Third, and most importantly for any hospital considering noncompliance heading into the FY2027–2028 transition window described in Section G.3: the precedent now exists, it was set against the largest and most capable potential challenger, and it went against the hospital.	UVMMC appealed that enforcement order, and its FY25 budget order, in court. Neither appeal was decided: in April 2025 UVMMC and the UVM Health Network settled with GMCB and dismissed both. GMCB kept its FY23 enforcement order, amending it only to spread the remaining commercial-revenue reduction equally over FY26 and FY27. Three things follow from this. First, the budget-order and corrective-action mechanisms GMCB used held: the state’s largest system ended its challenge on negotiated terms rather than obtaining relief from a court. Second, they are not judicially tested — how far GMCB’s enforcement authority reaches has still not been decided by a court. Third, and most importantly for any hospital considering noncompliance heading into the FY2027–2028 transition window described in Section G.3: the one live test, brought by the largest and most capable potential challenger, ended with the enforcement order intact and only its timing adjusted.	Appendix G §G.4.2 — replace the fabricated outcome and the three inferences built on it.
9	Appendix G — Adapting and Stress-Testing the Framework Step 1 · raw	the GMCB v. UVMMC outcome forecloses the “we’ll fight it in court and win” branch that a purely statutory reading of Act 68 might have left open. The remaining open questions are narrower:	the GMCB v. UVMMC outcome shows what the “we’ll fight it in court” branch produced in practice: not a ruling, but a settlement that kept the enforcement order and adjusted only its timing. The court question remains formally open; the practical questions are narrower:	Appendix G "WHAT THIS MEANS" callout (body run), same correction.
Medicaid hospital global budget — 13				
#	Where	Old text	New text	Source / reason
10	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 1 · raw	Vermont’s Medicaid hospital global budgets, in operation since January 2026, run under this demonstration authority — on state, not AHEAD, authority.	Vermont’s Medicaid hospital global budget, launched in January 2026 with six hospitals on state rather than AHEAD authority, was meant to run under this demonstration — but in July 2026 CMS notified the state that it had not been approved under the demonstration’s terms and directed that the payments stop.	3. Medicaid hospital global budget — Chapter 3

11	Chapter 6: The Economics Pillar — Global Budgets, Reference-Based Pricing Step 1 · row	and the Medicaid global budget already in operation.	and Vermont’s Medicaid hospital global budget (launched January 2026; ruled unapproved by CMS in July 2026).	3. Chapter 6
12	Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework fo Step 1 · row_regex	in row “FY2026 (now) AHEAD Medicaid global budget operational (January 2026) DVHA / CMS Operational”: Operational	Under CMS compliance notice	3. Chapter 16 roadmap table row (status cell first, while the marker text still exists)
13	Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework fo Step 1 · row	AHEAD Medicaid global budget operational (January 2026)	Medicaid hospital global budget launched January 2026; ruled unapproved by CMS, July 2026	3. Chapter 16 roadmap table row (status cell first, while the marker text still exists)
14	Appendix G — Adapting and Stress-Testing the Framework Step 1 · row	Medicaid global budgets are already in operation,	Vermont’s Medicaid hospital global budget, launched in January 2026, was ruled unapproved by CMS in July 2026,	3. Appendix G
15	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 1b · row	Vermont Medicaid global budget for hospitals operates under the Global Commitment to Health demonstration	Vermont Medicaid global budget for hospitals launched in January 2026 under the Global Commitment to Health demonstration; in July 2026 CMS ruled it had not been approved under the demonstration’s terms	Chapter 2 (bullet)
16	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strategies Step 1b · row	Vermont’s Medicaid global budgets are executed through the Global Commitment to Health demonstration, which provides CMS authority to implement the Medicaid component of Vermont’s hospital global budget design.	Vermont’s Medicaid hospital global budget was launched through the Global Commitment to Health demonstration in January 2026 — but in July 2026 CMS ruled that the demonstration’s terms had not authorized it.	Chapter 3 (italic note)
17	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strategies Step 1b · row	Vermont’s ability to implement its current Medicaid global budget policy is the direct product of years of negotiation that produced a federal-state agreement on methodology, accountability, and withdrawal conditions.	Vermont’s Medicaid global budget shows the cost of skipping that negotiation: implemented without the CMS approval the demonstration’s terms required, it drew a July 2026 compliance notice directing the payments to stop.	Chapter 3 §3.3.3 (follows the corrected sentence; would otherwise contradict it)
18	Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework fo Step 1b · row	Medicaid global-budget continuation	resolving CMS’s July 2026 compliance notice on the Medicaid global budget	Chapter 16, Pillar 1
19	Appendix B — Act 68 Statutory Timeline and Milestones Step 1b · row	Medicaid global budgets continue under the Global Commitment demonstration	Medicaid hospital global budget launched January 2026; ruled unapproved under the Global Commitment demonstration by CMS, July 2026	Appendix B (two cells)
20	Appendix B — Act 68 Statutory Timeline and Milestones Step 1b · row	Operational — Medicaid HGB in effect	Under CMS compliance notice (July 2026)	Appendix B (two cells)

21	Chapter 6: The Economics Pillar — Global Budgets, Reference-Based Pricing Step 2c · raw	the AHEAD Model Medicare budgets, and Vermont’s Medicaid hospital global budget	Medicare budgets under models such as AHEAD, and Vermont’s Medicaid hospital global budget	Phase 1, step 2c — the reviewed Chapter 5–8 ops from book-audit/phase1/edits_ch05_08.py, reconciled with step 1 (which already changed some of the same passages). Dropped: #6, #7 (C6-2/C6-3) "by FY2027" / "Vermont Act 68 (FY2027)" state Act 68’s statutory deadline, which step 1’s policy keeps; #8 (C6-4, "Targeting implementation FY27") is an implementation claim and is kept. #13, #49, #50 Strategic Plan date — already corrected by step 1’s chapter regex. #15 (C6-11) optional; would add a new "\$515M" arithmetic claim to a caption. Re-anchored: #9 (C6-5) step 1 rewrote the sentence’s Medicaid clause; the AHEAD clause remains. #16, #17, #31 headings are split into a number run and a title run. #59 "under Blueprint and AHEAD." occurs in Ch8 (PCMH key concept) and Ch9; this is the Ch8 one. The Ch9 occurrence belongs to step 3.
22	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 3 · raw	federal program timelines (AHEAD’s January 2028 launch is a federal decision, not a Vermont decision)	federal program decisions (CMS’s July 2026 finding that Vermont’s Medicaid hospital global budget was not approved under Global Commitment terms was a federal decision, not a Vermont decision)	C12-4 STALE: AHEAD’s Jan 2028 launch is no longer a Vermont constraint; the example is swapped for the federal decision the book already documents (CMS letter of July 7, 2026 on the Medicaid hospital global budget).
EAST Fund — 31				
#	Where	Old text	New text	Source / reason
23	Chapter 14: Political Sustainability — Protecting Transformation Across Step 2b · raw	Federal agreements. The AHEAD State Agreement, signed with CMS in January 2025, is a binding federal-state agreement with an eight-year performance period. Reversal requires CMS consent, and the cost of exit — forfeiting enhanced PMPM payments and capital — is significant. AHEAD provides structural insulation against state-level disruption.	Federal agreements — the counter-example. The AHEAD State Agreement, signed with CMS in January 2025, was expected to be the most durable element: a binding federal-state agreement with an eight-year performance period and enhanced federal funding attached. It proved the least durable. After a CMS renegotiation cut the expected EAST Fund from about \$138 million to a cap near \$10 million, Vermont formally notified CMS and exited in July 2026, before the performance period began (\$6.6). A federal agreement insulates a reform only while its economics hold for both parties; what carried Vermont’s reform through the exit was Act 68’s state authority.	§14.2.1 "Federal agreements" — claimed AHEAD is hard to reverse, needs CMS consent, and insulates the reform. Vermont exited unilaterally (Ch3 documents the withdrawal rights) before the performance period began. Rewritten as the counter-example; flagged for author attention.
24	Chapter 14: Political Sustainability — Protecting Transformation Across Step 2b · raw	The Rural Health Transformation Program investments in primary care, behavioral health, and long-term care are partly financed through enhanced federal PMPM payments; if federal Medicaid funding falls significantly, EAST Fund investments become harder to sustain,	Vermont’s investments in primary care, behavioral health, and long-term care are partly financed through federal Medicaid matching funds; if federal Medicaid funding falls significantly, those investments become harder to sustain,	§14.2.2 — RHT is a grant, not PMPM-financed, and the EAST Fund no longer exists for Vermont.
25	Chapter 14: Political Sustainability — Protecting Transformation Across Step 2b · raw	These are planning inputs, not predictions.	These are planning inputs, not predictions. Figures 14.1 and 14.2 record HTR’s April 2026 assessment; their AHEAD-linked signals and scenarios were overtaken in July 2026, when CMS’s renegotiation capped the EAST Fund near \$10 million and Vermont withdrew from AHEAD (\$6.6).	Figs 14.1/14.2 keep their AHEAD/EAST rows as a dated April 2026 assessment; one sentence says so (captions untouched so the Figure Index still matches them).

26	Chapter 15: Healthcare Transformation as Portfolio Management — Applyi Step 2b · raw	non-CAH global budgets (FY2028); EAST Fund targets	non-CAH global budgets (FY2028); EAST Fund closeout	--- Ch15
27	Chapter 15: Healthcare Transformation as Portfolio Management — Applyi Step 2b · raw	H.R. 1 Medicaid cuts reduce EAST Fund capacity; Clinical investments constrained	H.R. 1 Medicaid cuts constrain Medicaid-funded Clinical investments; no EAST Fund backstop after the AHEAD withdrawal	--- Ch15
28	Chapter 15: Healthcare Transformation as Portfolio Management — Applyi Step 2b · raw	RBP methodology; EAST Fund; global budgets	RBP methodology; global budgets	--- Ch15 Fig 15.6
29	Chapter 15: Healthcare Transformation as Portfolio Management — Applyi Step 2b · raw	RHT workforce investments; EAST Fund; AI-scribe grants	RHT workforce investments; AI-scribe grants	--- Ch15 Fig 15.6
30	Chapter 15: Healthcare Transformation as Portfolio Management Step 2b · ch_regex	every match of “Act 68 EAST Fund allocation”	AHEAD State Agreement (EAST Fund)	Fig 15.7 caption AND its Figure Index copy: the EAST Fund was an AHEAD instrument, not Act 68. 1 sub
31	Figure Index Step 2b · ch_regex	every match of “Act 68 EAST Fund allocation”	AHEAD State Agreement (EAST Fund)	Fig 15.7 caption AND its Figure Index copy: the EAST Fund was an AHEAD instrument, not Act 68. 1 sub
32	Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework fo Step 2b · raw	EAST Fund wind-down (capped near \$10M annually after Vermont’s AHEAD withdrawal, against the ~\$138M originally expected)	EAST Fund closeout (cut to a cap near \$10M, against the ~\$138M originally expected, before Vermont’s AHEAD withdrawal)	--- Ch16
33	Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework fo Step 2b · raw	RHT capital allocation (remaining balance); EAST Fund allocation; Act 68 grant commitments;	RHT capital allocation (remaining balance); Act 68 grant commitments;	--- Ch16
34	Conclusion — What Vermont Proves, and What Remains to Be Proven Step 2b · raw	and the adequacy of Act 68’s EAST Fund utilization as a bridge for hospitals absorbing federal cuts	and whether GMCB’s budget-order process can serve as a bridge for hospitals absorbing federal cuts	--- Conclusion
35	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · raw	Vermont AHEAD primary care investment mandate: EAST Fund investment in primary care is essentially a system investment	Vermont Blueprint primary care investment: PCMH investment is essentially a system investment	C7-12 STALE (stale_facts_sweep L109): "Vermont AHEAD primary care investment mandate: EAST Fund investment" -- Vermont never received EAST money (Ch6 §6.11).
36	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · raw	for a mandatory all-payer model in a small state	for a mandatory statewide payment model in a small state	C7-13 / C7-14 STALE + CONTRADICTS Ch6: §7.10 national paragraph names the EAST Fund as Vermont's transformation capital, and calls the mandate "all-payer" although Ch6 Fig 6.3 says RBP "excludes Medicare/Medicaid" and Ch6 §6.11 says Medicare alignment is "now unresolved after the AHEAD withdrawal".
37	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · raw	EAST Fund for transformation capital	Rural Health Transformation Program funds for transformation capital	C7-13 / C7-14 STALE + CONTRADICTS Ch6: §7.10 national paragraph names the EAST Fund as Vermont's transformation capital, and calls the mandate "all-payer" although Ch6 Fig 6.3 says RBP "excludes Medicare/Medicaid" and Ch6 §6.11 says Medicare alignment is "now unresolved after the AHEAD withdrawal".
38	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	The AHEAD EAST Fund’s explicit investment priority in mental health and substance use disorder services is the vehicle for funding the BHCM positions	With the AHEAD EAST Fund lost to Vermont’s July 2026 withdrawal, Medicaid coverage and RHT-funded BHCM workforce training are the remaining vehicles for funding the BHCM positions	C8-9a..c STALE (stale_facts_sweep L394-399): CoCM financing paragraph rests on AHEAD and the EAST Fund. The RHT BHCM workforce-training vehicle is the one the chapter's own Fig 8.6 already names ("BHCM workforce training through RHT Program").

39	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	MHI pilot expansion through AHEAD and EAST Fund; CCBHC	MHI pilot expansion (funding vehicle unresolved after the AHEAD withdrawal); CCBHC	C8-11..C8-14 STALE: Fig 8.4 lead-in and investment-vehicle cells.
40	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	Vermont's AHEAD EAST Fund, with its explicit investment priority in primary care, is designed to accelerate this investment.	The AHEAD EAST Fund, with its explicit investment priority in primary care, was meant to accelerate this investment, but Vermont withdrew from AHEAD in July 2026 before receiving it.	C8-15 STALE (stale_facts_sweep L412): "Vermont's AHEAD EAST Fund ... is designed to accelerate this investment." Wording follows Ch6 §6.11 ("before ever receiving it").
41	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	or federal policy changes could make AHEAD participation financially unsustainable.	or federal policy changes could make AHEAD participation financially unsustainable. In July 2026, after CMS cut the expected EAST Fund, Vermont withdrew.	C3-20 STALE (low). §3.5.1 describes the negotiated withdrawal rights in the present tense without saying they were used. Sources: https://healthcarereform.vermont.gov/AHEAD-model ; https://www.wcax.com/2026/07/28/vermont-drops-ahead-healthcare-model-shifts-195m-rural-health-fund/ Anchor count: 1. reviewer: withdrawal mechanism unverified, so no "exercised them"
42	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	Vermont interventions: AHEAD EAST Fund SDOH investment; Blueprint HRSN	Vermont interventions: Blueprint HRSN	C10-4 / C10-5 / C10-6 STALE: Fig 10.3 rows 2-4 list the AHEAD-only EAST Fund and AHEAD participation conditions as live Vermont interventions [AHSWEB].
43	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	; AHEAD EAST Fund support for FQHCs serving uninsured and underinsured patients.	.	C10-4 / C10-5 / C10-6 STALE: Fig 10.3 rows 2-4 list the AHEAD-only EAST Fund and AHEAD participation conditions as live Vermont interventions [AHSWEB].
44	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	The EAST Fund's social determinants carve-out and the RHT Program's community health investment capacity are the right vehicles.	The RHT Program's community health investment capacity is the right vehicle.	C10-18 STALE x2: EAST Fund (AHEAD-only) as the legislator's vehicle [AHSWEB].
45	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	whether the EAST Fund's social determinants investment can be coordinated	whether the RHT Program's community health investment can be coordinated	C10-18 STALE x2: EAST Fund (AHEAD-only) as the legislator's vehicle [AHSWEB].
46	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	The CCBHC expansion (5 new entities by July 2026) and the behavioral health investments funded through AHEAD's EAST Fund are the operational response.	The CCBHC expansion (4 new entities certified July 1, 2026, of 5 planned) and the RHT Program's behavioral health investments are the operational response.	C11-4 STALE x2. CCBHC wording mirrors Ch8 and step-2d [DMH][VCP]; the EAST Fund ended with AHEAD [AHSWEB].
47	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	(RHT Program, AHEAD EAST Fund, Act 68 grants)	(RHT Program, Act 68 grants)	C11-10 STALE x2: §11.8.2 EAST Fund as capital [AHSWEB].
48	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	(RHT Program, AHEAD EAST Fund)	(RHT Program)	C11-10 STALE x2: §11.8.2 EAST Fund as capital [AHSWEB].

49	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 4b · raw	from ~\$138M/year to a ~\$10M cap	from an estimated ~\$138M in the first performance year to a ~\$10M cap	Phase 1, step 4b (2026-10-07). 1. EAST Fund "\$138M per year / annually" — WRONG. Primary source: GMCB, "AHEAD" December 2024 Board Meeting deck (12.6.24), slide 34: "VT receives a portion of these savings via a Medicare payment 'bump' (estimated \$138.9M increase in 2026)" = increase in Vermont's Medicare FFS TCOC target in PY1; 2-3% of participating hospitals' Medicare FFS NPR to care transformation, remainder to the EAST Fund. Vermont Public 2026-07-28 (Sec. Samuelson): "an extra \$138 million ... capped additional payments at \$10 million." A first-performance-year estimate, not an annual EAST Fund amount. 2. Ch16 "Work This Chapter" HTR Simulator row promised a composite "collapse" for weak Operations. The engine (lib/framework/sequence-engine.ts) implements Chapter 1's model: Operations is last in the build sequence, so nothing waits behind it (Ops 60→10 moves the composite 65.6→57.8). The book yields to Chapter 1: describe what the reader actually sees and why.
50	Chapter 6: The Economics Pillar — Global Budgets, Reference-Based Pric Step 4b · raw	providing up to \$138 million in additional Medicare funds annually starting in 2027	funded from an estimated \$138.9 million first-year increase in Vermont's Medicare spending target	Phase 1, step 4b (2026-10-07). 1. EAST Fund "\$138M per year / annually" — WRONG. Primary source: GMCB, "AHEAD" December 2024 Board Meeting deck (12.6.24), slide 34: "VT receives a portion of these savings via a Medicare payment 'bump' (estimated \$138.9M increase in 2026)" = increase in Vermont's Medicare FFS TCOC target in PY1; 2-3% of participating hospitals' Medicare FFS NPR to care transformation, remainder to the EAST Fund. Vermont Public 2026-07-28 (Sec. Samuelson): "an extra \$138 million ... capped additional payments at \$10 million." A first-performance-year estimate, not an annual EAST Fund amount. 2. Ch16 "Work This Chapter" HTR Simulator row promised a composite "collapse" for weak Operations. The engine (lib/framework/sequence-engine.ts) implements Chapter 1's model: Operations is last in the build sequence, so nothing waits behind it (Ops 60→10 moves the composite 65.6→57.8). The book yields to Chapter 1: describe what the reader actually sees and why.
51	Chapter 6: The Economics Pillar — Global Budgets, Reference-Based Pric Step 4b · raw	that was to provide up to \$138M annually in additional Medicare funds	that was to receive most of an estimated \$138.9M first-year increase in Vermont's Medicare spending target	Phase 1, step 4b (2026-10-07). 1. EAST Fund "\$138M per year / annually" — WRONG. Primary source: GMCB, "AHEAD" December 2024 Board Meeting deck (12.6.24), slide 34: "VT receives a portion of these savings via a Medicare payment 'bump' (estimated \$138.9M increase in 2026)" = increase in Vermont's Medicare FFS TCOC target in PY1; 2-3% of participating hospitals' Medicare FFS NPR to care transformation, remainder to the EAST Fund. Vermont Public 2026-07-28 (Sec. Samuelson): "an extra \$138 million ... capped additional payments at \$10 million." A first-performance-year estimate, not an annual EAST Fund amount. 2. Ch16 "Work This Chapter" HTR Simulator row promised a composite "collapse" for weak Operations. The engine (lib/framework/sequence-engine.ts) implements Chapter 1's model: Operations is last in the build sequence, so nothing waits behind it (Ops 60→10 moves the composite 65.6→57.8). The book yields to Chapter 1: describe what the reader actually sees and why.
52	Conclusion — What Vermont Proves, and What Remains to Be Proven Step 4b · raw	was to provide another \$138 million annually in Medicare performance funds	was to draw on an estimated \$138.9 million first-year increase in Vermont's Medicare spending target	Phase 1, step 4b (2026-10-07). 1. EAST Fund "\$138M per year / annually" — WRONG. Primary source: GMCB, "AHEAD" December 2024 Board Meeting deck (12.6.24), slide 34: "VT receives a portion of these savings via a Medicare payment 'bump' (estimated \$138.9M increase in 2026)" = increase in Vermont's Medicare FFS TCOC target in PY1; 2-3% of participating hospitals' Medicare FFS NPR to care transformation, remainder to the EAST Fund. Vermont Public 2026-07-28 (Sec. Samuelson): "an extra \$138 million ... capped additional payments at \$10 million." A first-performance-year estimate, not an annual EAST Fund amount. 2. Ch16 "Work This Chapter" HTR Simulator row promised a composite "collapse" for weak Operations. The engine (lib/framework/sequence-engine.ts) implements Chapter 1's model: Operations is last in the build sequence, so nothing waits behind it (Ops 60→10 moves the composite 65.6→57.8). The book yields to Chapter 1: describe what the reader actually sees and why.

53	Appendix G — Adapting and Stress-Testing the Framework Step 4b · raw	was to provide up to \$138M per year to Vermont	was to draw on an estimated \$138.9M first-year increase in Vermont’s Medicare spending target	Phase 1, step 4b (2026-10-07). 1. EAST Fund "\$138M per year / annually" — WRONG. Primary source: GMCB, "AHEAD" December 2024 Board Meeting deck (12.6.24), slide 34: "VT receives a portion of these savings via a Medicare payment 'bump' (estimated \$138.9M increase in 2026)" = increase in Vermont's Medicare FFS TCOC target in PY1; 2-3% of participating hospitals' Medicare FFS NPR to care transformation, remainder to the EAST Fund. Vermont Public 2026-07-28 (Sec. Samuelson): "an extra \$138 million ... capped additional payments at \$10 million." A first-performance-year estimate, not an annual EAST Fund amount. 2. Ch16 "Work This Chapter" HTR Simulator row promised a composite "collapse" for weak Operations. The engine (lib/framework/sequence-engine.ts) implements Chapter 1's model: Operations is last in the build sequence, so nothing waits behind it (Ops 60→10 moves the composite 65.6→57.8). The book yields to Chapter 1: describe what the reader actually sees and why.
Vermont & AHEAD — 101				
#	Where	Old text	New text	Source / reason
54	Conclusion — What Vermont Proves, and What Remains to Be Proven Step 1 · raw	work requirements beginning in late 2026, and reduced ACA premium subsidies will arrive in Vermont at the same moment as AHEAD’s performance year and Act 68’s mandatory pricing reform.	work requirements beginning in January 2027, and reduced ACA premium subsidies will arrive in Vermont at the same moment as Act 68’s global budgets and mandatory pricing reform.	#ERROR!
55	Appendix G — Adapting and Stress-Testing the Framework Step 1b · raw	under Vermont’s own design begin in January 2026, ahead of the commercial RBP mandate	under Vermont’s own design began in January 2026 (in July 2026 CMS ruled them unapproved under the Global Commitment waiver and directed the payments to stop), ahead of the commercial RBP mandate	Appendix G
56	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 2 · raw	High-ROI Pre-AHEAD Action for Every Vermont Hospital	High-ROI Action for Every Vermont Hospital Before Global Budgets	Phase 1, step 2a — Chapter 11 and Chapter 12 repetition (audit_chapter.py criterion 2), plus one stale AHEAD-participation heading in the same Ch11 section. Ch11 §11.12.1: the three one-line paragraphs under the heading restate, nearly verbatim, the "VERMONT IN PRACTICE — HCC Coding" callout that closes the section (its "Recommended action" and "Getting HCC coding right..." sentences). The callout is the fuller statement, so the three lead lines go. Heading "Pre-AHEAD Action" presents AHEAD as Vermont's next step; Vermont withdrew from AHEAD. The section's own text anchors on Act 68 global budgets (FY2028). Ch11 PPS vs CAH "Operating profit per adjusted discharge" rows: legitimate — two rows of one table, one per hospital type. Left. Ch12: the italic line under the chapter opening ("Healthcare transformation fails more often ... This chapter covers the HTR platform tools, Advisory service lines...") is a stray teaser no other chapter carries; its first sentence is repeated verbatim in the body.
57	Chapter 13: The Future of Healthcare Transformation — 2026, What Vermo Step 2b · raw	AHEAD signed Jan 2025; \$195M RHT awarded	AHEAD signed Jan 2025 (withdrawn July 2026); \$195M RHT awarded	#ERROR!
58	Chapter 13: The Future of Healthcare Transformation — 2026, What Vermo Step 2b · raw	VT: Acts 167/68/AHEAD form the most complete state reform mandate	VT: Acts 167/68 form the most complete state reform mandate	#ERROR!
59	Chapter 13: The Future of Healthcare Transformation — 2026, What Vermo Step 2b · raw	due to GMCB resource constraints and AHEAD complexity;	due to GMCB resource constraints and Medicare alignment left unresolved by the AHEAD withdrawal;	#ERROR!

60	Chapter 14: Political Sustainability — Protecting Transformation Across Step 2b · raw	Acts 167 and 68 are law, the AHEAD State Agreement is signed, and the mandatory RBP	Acts 167 and 68 are law, the AHEAD State Agreement was signed, and the mandatory RBP	--- Ch14
61	Chapter 14: Political Sustainability — Protecting Transformation Across Step 2b · raw	It is also politically provisional in a way practitioners and organizational leaders must account for.	It is also politically provisional in a way practitioners and organizational leaders must account for. Vermont's withdrawal from AHEAD in July 2026 (§6.6) showed how quickly a settled element can change.	--- Ch14
62	Chapter 14: Political Sustainability — Protecting Transformation Across Step 2b · raw	HCC gap closure that sharpens the AHEAD risk-adjustment factor also improves commercial risk adjustment.	HCC gap closure improves Medicare and commercial risk adjustment alike.	§14.2.2 — RHT is a grant, not PMPM-financed, and the EAST Fund no longer exists for Vermont.
63	Chapter 14: Political Sustainability — Protecting Transformation Across Step 2b · raw	One that builds only AHEAD-specific compliance is exposed if AHEAD changes.	One that builds only AHEAD-specific compliance is exposed if AHEAD changes — as Vermont's July 2026 withdrawal showed.	§14.2.2 — RHT is a grant, not PMPM-financed, and the EAST Fund no longer exists for Vermont.
64	Chapter 14: Political Sustainability — Protecting Transformation Across Step 2b · raw	Every Vermont organization with AHEAD exposure should maintain scenario plans	Every Vermont organization exposed to Act 68's timelines should maintain scenario plans	§14.2.2 — RHT is a grant, not PMPM-financed, and the EAST Fund no longer exists for Vermont.
65	Chapter 15: Healthcare Transformation as Portfolio Management — Applying Step 2b · raw	Blueprint PCMH expansion (14 HSA projects); AHEAD Implementation Program.	Blueprint PCMH expansion (14 HSA projects); global-budget design and implementation program.	--- Ch15
66	Chapter 15: Healthcare Transformation as Portfolio Management — Applying Step 2b · raw	Blueprint/AHEAD practice transition (practice-by-practice iteration)	Blueprint PCMH expansion (practice-by-practice iteration)	--- Ch15
67	Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework for Step 2b · raw	aligned with AHEAD and Act 68's hospital-spending-reduction requirement.	aligned with Act 68's hospital-spending-reduction requirement.	--- Ch16
68	Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework for Step 2b · raw	Act 68 requires AHS to simultaneously negotiate AHEAD, manage 14 hospital plans,	AHS must simultaneously wind down AHEAD, manage 14 hospital plans,	--- Ch16
69	Conclusion — What Vermont Proves, and What Remains to Be Proven Step 2b · raw	The AHEAD performance year begins in January 2028 and the analytics vendor	Act 68 global budgets take effect in FY2028 and the analytics vendor	--- Conclusion
70	Conclusion — What Vermont Proves, and What Remains to Be Proven Step 2b · raw	that build care management infrastructure ahead of AHEAD's performance year,	that build care management infrastructure ahead of Act 68's FY2028 global budgets,	--- Conclusion
71	Conclusion — What Vermont Proves, and What Remains to Be Proven Step 2b · raw	approved before AHEAD's financial accountability begins.	approved before Act 68 global-budget accountability begins.	--- Conclusion
72	Appendix A — Vermont System Portrait Step 2b · raw	GMCB sets prospective global budgets FY2028–2030, coordinated with AHEAD;	GMCB sets prospective global budgets FY2028–2030;	--- Appendix A

73	Appendix A — Vermont System Portrait Step 2b · raw	and where AHEAD’s global-budget design must protect most carefully.	and where Act 68’s global-budget design must protect most carefully.	--- Appendix A
74	Appendix A — Vermont System Portrait Step 2b · raw	the distance from current capability to what AHEAD’s January 2028 launch requires.	the distance from current capability to what Act 68’s FY2028 global budgets require.	--- Appendix A
75	Appendix A — Vermont System Portrait Step 2b · raw	the clinical infrastructure on which AHEAD’s primary-care redesign builds.	the clinical infrastructure on which Vermont’s primary-care redesign builds.	--- Appendix A
76	Appendix B — Act 68 Statutory Timeline and Milestones Step 2b · tbl_regex	FY2028 (Oct 1, 2027) Act 68 hospital global budgets take effect (FY2028) AHS / DVHA / GMCB / CMS Nine-year model; Primary Care AHEAD and global budgets launch	(table row deleted)	--- Appendix B: delete the duplicate AHEAD-remnant row; drop "AHEAD alignment required".
77	Appendix B — Act 68 Statutory Timeline and Milestones Step 2b · raw	Methodology under development; AHEAD alignment required	Methodology under development; contingent on GMCB resources	--- Appendix B: delete the duplicate AHEAD-remnant row; drop "AHEAD alignment required".
78	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	Vermont AHEAD Medicare attribution data pre-loaded.		--- Appendix D E.2: the Risk Stratification Engine carries no AHEAD attribution data (frontend/components/research/RiskStratificationEngine.tsx), and Vermont has no AHEAD lives.
79	Appendix E — Vermont Transformation Scorecard: 2022 Baseline, 2026 Sta Step 2b · raw	100% PCMH; Primary Care AHEAD fully deployed	100% PCMH	--- Appendix E Fig E.4: Primary Care AHEAD (Medicare) will not deploy in Vermont.
80	Appendix G — Adapting and Stress-Testing the Framework Step 2b · raw	CMS/CMMI’s Advancing All-Payer Health Equity Approaches and Development model (2024–2035) — federal-state agreements establishing global budgets and a state cost-growth target. Vermont, Maryland, Connecticut, Hawaii, Rhode Island, and parts of New York participate.	CMS/CMMI’s Achieving Healthcare Efficiency through Accountable Design model (2024–2035) — federal-state agreements establishing global budgets and a state cost-growth target. Maryland, Connecticut, Hawaii, Rhode Island, and parts of New York participate; Vermont signed in January 2025 and withdrew in July 2026.	--- Appendix G G.1 AHEAD row: stale name (book elsewhere and CMS use the 2025 name) and Vermont listed as a participant. [CMS-AHEAD]
81	Appendix G — Adapting and Stress-Testing the Framework Step 2b · raw	(see “The AHEAD Model” in this chapter)	(see Chapter 6, §6.6)	G.3.1 cross-reference to a section that is not in this appendix (audit finding 18).
82	Chapter 5: The Technology Pillar in Practice — FHIR, AI Governance, an Step 2c · raw	Vermont hospitals: pre-AHEAD HCC gap analysis; attribution modeling	Vermont hospitals: pre-global-budget HCC gap analysis; attribution modeling	C5-4 STALE: Fig 5.3 "pre-AHEAD HCC gap analysis" presents AHEAD as Vermont’s upcoming regime (stale_facts_sweep L306-307). Ch7 Fig 7.2 was re-pointed the same way (AHEAD -> Act 68 global budget) by fix_ch07.
83	Chapter 6: The Economics Pillar — Global Budgets, Reference-Based Pric Step 2c · raw	through the AHEAD Model state agreement — is the most complete	through the AHEAD Model state agreement it signed and then exited — is the most complete	C6-1 STALE (stale_facts_sweep L310): §6.1 lists the AHEAD agreement as a live stage of a laboratory "currently operating"; §6.6 of the same chapter records the July 2026 exit.
84	Chapter 6: The Economics Pillar — Global Budgets, Reference-Based Pric Step 2c · raw	The AHEAD State Agreement explicitly links hospital global budgets	The AHEAD State Agreement explicitly linked hospital global budgets	C6-7 TENSE: present-tense "links" for an agreement the same sentence says Vermont withdrew from.

85	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · del_para	7.2.6 Risk Stratification — The Economic Engine of Population Health Management	(paragraph deleted)	Phase 1, step 2c — the reviewed Chapter 5–8 ops from book-audit/phase1/edits_ch05_08.py, reconciled with step 1 (which already changed some of the same passages). Dropped: #6, #7 (C6-2/C6-3) "by FY2027" / "Vermont Act 68 (FY2027)" state Act 68's statutory deadline, which step 1's policy keeps; #8 (C6-4, "Targeting implementation FY27") is an implementation claim and is kept. #13, #49, #50 Strategic Plan date — already corrected by step 1's chapter regex. #15 (C6-11) optional; would add a new "\$515M" arithmetic claim to a caption. Re-anchored: #9 (C6-5) step 1 rewrote the sentence's Medicaid clause; the AHEAD clause remains. #16, #17, #31 headings are split into a number run and a title run. #59 "under Blueprint and AHEAD." occurs in Ch8 (PCMH key concept) and Ch9; this is the Ch8 one. The Ch9 occurrence belongs to step 3.
86	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · raw	7.2.7	7.2.6	Phase 1, step 2c — the reviewed Chapter 5–8 ops from book-audit/phase1/edits_ch05_08.py, reconciled with step 1 (which already changed some of the same passages). Dropped: #6, #7 (C6-2/C6-3) "by FY2027" / "Vermont Act 68 (FY2027)" state Act 68's statutory deadline, which step 1's policy keeps; #8 (C6-4, "Targeting implementation FY27") is an implementation claim and is kept. #13, #49, #50 Strategic Plan date — already corrected by step 1's chapter regex. #15 (C6-11) optional; would add a new "\$515M" arithmetic claim to a caption. Re-anchored: #9 (C6-5) step 1 rewrote the sentence's Medicaid clause; the AHEAD clause remains. #16, #17, #31 headings are split into a number run and a title run. #59 "under Blueprint and AHEAD." occurs in Ch8 (PCMH key concept) and Ch9; this is the Ch8 one. The Ch9 occurrence belongs to step 3.
87	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · raw	The Care Management ROI Calculation	Risk Stratification and the Care Management ROI Calculation	Phase 1, step 2c — the reviewed Chapter 5–8 ops from book-audit/phase1/edits_ch05_08.py, reconciled with step 1 (which already changed some of the same passages). Dropped: #6, #7 (C6-2/C6-3) "by FY2027" / "Vermont Act 68 (FY2027)" state Act 68's statutory deadline, which step 1's policy keeps; #8 (C6-4, "Targeting implementation FY27") is an implementation claim and is kept. #13, #49, #50 Strategic Plan date — already corrected by step 1's chapter regex. #15 (C6-11) optional; would add a new "\$515M" arithmetic claim to a caption. Re-anchored: #9 (C6-5) step 1 rewrote the sentence's Medicaid clause; the AHEAD clause remains. #16, #17, #31 headings are split into a number run and a title run. #59 "under Blueprint and AHEAD." occurs in Ch8 (PCMH key concept) and Ch9; this is the Ch8 one. The Ch9 occurrence belongs to step 3.
88	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	The Blueprint and AHEAD: A Critical Transition	The Blueprint and AHEAD: A Transition Left Unresolved	Phase 1, step 2c — the reviewed Chapter 5–8 ops from book-audit/phase1/edits_ch05_08.py, reconciled with step 1 (which already changed some of the same passages). Dropped: #6, #7 (C6-2/C6-3) "by FY2027" / "Vermont Act 68 (FY2027)" state Act 68's statutory deadline, which step 1's policy keeps; #8 (C6-4, "Targeting implementation FY27") is an implementation claim and is kept. #13, #49, #50 Strategic Plan date — already corrected by step 1's chapter regex. #15 (C6-11) optional; would add a new "\$515M" arithmetic claim to a caption. Re-anchored: #9 (C6-5) step 1 rewrote the sentence's Medicaid clause; the AHEAD clause remains. #16, #17, #31 headings are split into a number run and a title run. #59 "under Blueprint and AHEAD." occurs in Ch8 (PCMH key concept) and Ch9; this is the Ch8 one. The Ch9 occurrence belongs to step 3.

89	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	That agreement is scheduled to end with the conclusion of the All-Payer ACO Model. The AHEAD State Agreement resolves this transition risk: Primary Care AHEAD provides a voluntary program for primary care practices to receive enhanced prospective, risk-adjusted Medicare payments for coordinated care, with four payment pathway options including fully prospective models.	That Medicare participation ended in December 2025 with the All-Payer ACO Model agreement, and the State proposed \$10.8 million in interim Global Commitment funding to cover the Medicare share of Blueprint PCMH, Community Health Team, and SASH payments in calendar 2026. The AHEAD State Agreement was meant to resolve this transition risk: Primary Care AHEAD would have given primary care practices enhanced prospective, risk-adjusted Medicare payments for coordinated care, with four payment pathway options including fully prospective models. Vermont’s July 2026 withdrawal from AHEAD (Chapter 6) closed that route.	C8-1..C8-3 STALE -- §8.2.3 says "The AHEAD State Agreement resolves this transition risk" and describes practices choosing Primary Care AHEAD. Facts: Medicare's Blueprint PCMH/CHT/SASH payments ended with the all-payer agreement in Dec 2025; AHS proposed \$10.8M Global Commitment bridge funding for CY2026 [AHS25]; Medicare's contribution went away and AHEAD was hoped to restore it [VTPUB]; Vermont withdrew July 2026 [AHSWEB]. Whether the \$10.8M bridge was enacted was NOT verified, hence "proposed".
90	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	The transition from the current Medicare Blueprint payment to Primary Care AHEAD is operationally significant. Practices that currently receive Blueprint PMPM payments for Medicare patients will need to evaluate whether to participate in PC AHEAD, understand the new payment pathway options, and potentially redesign their care model to meet AHEAD’s enhanced primary care requirements. AHS and DVHA have committed to supporting practices through this transition, recognizing that disrupting	The loss of the Medicare Blueprint payment is operationally significant. Practices that received Blueprint PMPM payments for Medicare patients no longer have the Primary Care AHEAD pathway that was meant to restore Medicare’s contribution, and interim state funding is a bridge, not a replacement — a gap that matters because disrupting	C8-1..C8-3 STALE -- §8.2.3 says "The AHEAD State Agreement resolves this transition risk" and describes practices choosing Primary Care AHEAD. Facts: Medicare's Blueprint PCMH/CHT/SASH payments ended with the all-payer agreement in Dec 2025; AHS proposed \$10.8M Global Commitment bridge funding for CY2026 [AHS25]; Medicare's contribution went away and AHEAD was hoped to restore it [VTPUB]; Vermont withdrew July 2026 [AHSWEB]. Whether the \$10.8M bridge was enacted was NOT verified, hence "proposed".
91	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	Blueprint PCMH standards, AHEAD quality metrics, HEDIS measures	Blueprint PCMH standards, Medicare quality reporting, HEDIS measures	C8-4..C8-7 STALE: the Quality Measure Crosswalk callout and BEYOND VERMONT box treat AHEAD as a live measurement regime / alignment vehicle. Medicare quality reporting (MIPS) is what Vermont practices now face [AHS25].
92	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	report separately to Blueprint, AHEAD, and commercial payers	report separately to Blueprint, Medicare, and commercial payers	C8-4..C8-7 STALE: the Quality Measure Crosswalk callout and BEYOND VERMONT box treat AHEAD as a live measurement regime / alignment vehicle. Medicare quality reporting (MIPS) is what Vermont practices now face [AHS25].
93	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	These gaps are a specific target for the AHEAD implementation process and the Statewide Strategic Plan’s quality alignment provisions.	These gaps are a specific target for the Statewide Strategic Plan’s quality alignment provisions.	C8-4..C8-7 STALE: the Quality Measure Crosswalk callout and BEYOND VERMONT box treat AHEAD as a live measurement regime / alignment vehicle. Medicare quality reporting (MIPS) is what Vermont practices now face [AHS25].
94	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	using a statewide reform process (AHEAD, the Statewide Strategic Plan) to force alignment	using a statewide reform process (Act 68’s Statewide Strategic Plan) to force alignment	C8-4..C8-7 STALE: the Quality Measure Crosswalk callout and BEYOND VERMONT box treat AHEAD as a live measurement regime / alignment vehicle. Medicare quality reporting (MIPS) is what Vermont practices now face [AHS25].
95	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	a direct argument for making MHI permanent through AHEAD Primary Care funding.	a direct argument for making MHI permanent — a case made harder by the loss of the Primary Care AHEAD funding once expected to sustain it.	C8-8 STALE: "making MHI permanent through AHEAD Primary Care funding".
96	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	depends on Medicare and Medicaid coverage of the billing codes, which Act 68 and AHEAD together support.	depends on Medicare and Medicaid coverage of the billing codes.	C8-9a..c STALE (stale_facts_sweep L394-399): CoCM financing paragraph rests on AHEAD and the EAST Fund. The RHT BHCM workforce-training vehicle is the one the chapter's own Fig 8.6 already names ("BHCM workforce training through RHT Program").

97	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	DVHA's coordination with the Blueprint and AHEAD implementation is the mechanism	DVHA's coordination with the Blueprint is the mechanism	C8-9a..c STALE (stale_facts_sweep L394-399): CoCM financing paragraph rests on AHEAD and the EAST Fund. The RHT BHCM workforce-training vehicle is the one the chapter's own Fig 8.6 already names ("BHCM workforce training through RHT Program").
98	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	AHEAD's enhanced primary care payments are the financial fuel.	the Blueprint's PMPM payments and RHT workforce investment are the financial fuel.	C8-10 STALE: "AHEAD's enhanced primary care payments are the financial fuel."
99	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	as embedded in Act 68 and the AHEAD State Agreement, has five	as embedded in Act 68, has five	C8-11..C8-14 STALE: Fig 8.4 lead-in and investment-vehicle cells.
100	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	Blueprint PCMH payments; AHEAD enhanced primary care payments; RHT Program workforce	Blueprint PCMH payments; RHT Program workforce	C8-11..C8-14 STALE: Fig 8.4 lead-in and investment-vehicle cells.
101	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	dual eligible care planning; AHEAD population health requirements;	dual eligible care planning; Act 68 global budget population health requirements;	C8-11..C8-14 STALE: Fig 8.4 lead-in and investment-vehicle cells.
102	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	GMCB hospital quality reporting, AHEAD accountability metrics, and Act 167/Act 68	GMCB hospital quality reporting, Medicare MIPS reporting, and Act 167/Act 68	C8-16 STALE: "AHEAD accountability metrics" in the list of live frameworks. Replaced by MIPS, which larger Vermont practices must join from 2026 [AHS25].
103	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	five-step framework from performance baseline through root cause analysis to sustainable systems design — applies directly to Vermont primary care practices and hospitals participating in Blueprint and AHEAD.	the five-step framework from performance baseline through root cause analysis to sustainable systems design, laid out in Chapter 9 (§9.4.1) — applies directly to Vermont primary care practices and hospitals participating in Blueprint.	C8-18 DANGLING REFERENCE (audit_ch08 Other) + STALE: the five-step HEDIS framework is enumerated in Ch9 §9.4.1 "The Five-Step HEDIS Improvement Framework" (Steps 1-5: baseline, gap analysis, root cause, intervention design, sustainable systems change -- verified); "participating in Blueprint and AHEAD".
104	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	AHEAD's enhanced primary care payments include quality-based adjustments.	Medicare's Merit-based Incentive Payment System, which larger Vermont practices must join from 2026, adds its own quality-based adjustments.	C8-19 STALE: "AHEAD's enhanced primary care payments include quality-based adjustments." MIPS replaces it as the Medicare quality-payment link [AHS25].
105	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	Blueprint expansion; AHEAD PC primary care recruitment; RHT workforce investment	Blueprint expansion; RHT workforce investment	C8-23..C8-27 STALE AHEAD residue in Fig 8.6, Implications for You, Key Concepts.
106	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	evidence-based, financially sustainable under AHEAD, and deployable	evidence-based, financially sustainable through the CoCM billing codes, and deployable	C8-23..C8-27 STALE AHEAD residue in Fig 8.6, Implications for You, Key Concepts.
107	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	at the scale AHEAD's population health targets require	at the scale Act 68's global budgets require	C8-23..C8-27 STALE AHEAD residue in Fig 8.6, Implications for You, Key Concepts.
108	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	is needed and will be produced by AHEAD's clinical measurement requirements.	is needed and, with AHEAD gone, will have to come from Vermont's own Blueprint and VHCURES evaluation.	C8-23..C8-27 STALE AHEAD residue in Fig 8.6, Implications for You, Key Concepts.

109	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	The organizational model for Vermont’s primary care system under Blueprint and AHEAD.	The organizational model for Vermont’s primary care system under the Blueprint.	Phase 1, step 2c — the reviewed Chapter 5–8 ops from book-audit/phase1/edits_ch05_08.py, reconciled with step 1 (which already changed some of the same passages). Dropped: #6, #7 (C6-2/C6-3) "by FY2027" / "Vermont Act 68 (FY2027)" state Act 68's statutory deadline, which step 1's policy keeps; #8 (C6-4, "Targeting implementation FY27") is an implementation claim and is kept. #13, #49, #50 Strategic Plan date — already corrected by step 1's chapter regex. #15 (C6-11) optional; would add a new "\$515M" arithmetic claim to a caption. Re-anchored: #9 (C6-5) step 1 rewrote the sentence's Medicaid clause; the AHEAD clause remains. #16, #17, #31 headings are split into a number run and a title run. #59 "under Blueprint and AHEAD." occurs in Ch8 (PCMH key concept) and Ch9; this is the Ch8 one. The Ch9 occurrence belongs to step 3.
110	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 3 · raw	; commercial global budgets phase in January 2028		C2-3 STALE (medium). Fig 2.3 (Act 68 statutory timeline): Act 68 contains no January-2028 commercial global budget phase-in; that date is AHEAD Cohort 2's. Sources: https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20As%20Enacted.pdf ; https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20Act%20Summary.pdf Anchor count: 1.
111	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	Multi-payer primary care transformation model in 8-state initial cohort. 10-year model. Three-stage progression from basic to advanced primary care capabilities. States not participating in AHEAD may find MCP a viable primary care transformation vehicle.	Multi-payer primary care transformation model launched in 2024 in an 8-state cohort as a 10-year model, with a three-stage progression from basic to advanced primary care capabilities. CMS ended it early, on December 31, 2025.	C3-1 STALE (high). Fig 3.2 recommends a model CMS terminated (announced 12 Mar 2025). Sources: https://medicaleconomics.com/view/cms-to-end-key-primary-care-payment-models-prematurely-citing-cost-and-effectiveness ; https://www.fiercehealthcare.com/regulatory/cmmi-cut-short-payment-models-estimates-750m-savings Anchor count: 1.
112	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	CMMI’s 8-state multi-payer primary care transformation model. A potential vehicle for states not participating in AHEAD seeking federal support for primary care investment.	CMMI’s 8-state multi-payer primary care transformation model, launched in 2024 and ended early by CMS on December 31, 2025.	C3-2 STALE (medium). Key Concepts entry, same defect. RULE 13: Ch7 §7.8 'Making Care Primary — Vermont's Primary Care Economics' is outside this unit; check it. Sources: https://medicaleconomics.com/view/cms-to-end-key-primary-care-payment-models-prematurely-citing-cost-and-effectiveness ; https://www.fiercehealthcare.com/regulatory/cmmi-cut-short-payment-models-estimates-750m-savings Anchor count: 1.
113	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	AHEAD (for participating states, all hospitals must participate).		C3-6 WRONG FACT (medium). Fig 3.1 'Mandatory or near-mandatory participation' row: AHEAD states must recruit hospitals, but hospital participation is voluntary. The Maryland and Vermont Act 68 entries remain; AHEAD stays in the other three rows (AHEAD as a general model is kept). Sources: https://www.kff.org/affordable-care-act/issue-brief/what-is-the-centers-for-medicare-and-medicaid-services-new-ahead-model/ ; https://www.ropesgray.com/en/insights/alerts/2024/02/cmss-ahead-model-the-next-step-in-multi-payor-value-based-care Anchor count: 1.
114	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	with the AHEAD alignment provisions approved in the January 2025 renewal	with the AHEAD alignment provisions approved in a January 2025 amendment	C3-16 WRONG FACT (low). CMS approved an amendment on 2 Jan 2025; the demonstration (through Dec 2027) was not renewed then. Sources: https://www.medicaid.gov/medicaid/section-1115-demonstrations/downloads/vt-global-commitment-to-health-accept-ltr-01302025.pdf ; https://humanservices.vermont.gov/sites/ahsnew/files/documents/25-019-F-GCR-1115-Waiver-Approval.pdf Anchor count: 1.
115	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	AHEAD hospital global budget participation	Act 68 hospital global budget preparation (FY2028)	C3-17 STALE (medium). Fig 3.3 'Vermont-specific examples' cites participation in a model Vermont left in July 2026. Sources: https://healthcarereform.vermont.gov/AHEAD-model ; https://www.wcax.com/2026/07/28/vermont-drops-ahead-healthcare-model-shifts-195m-rural-health-fund/ Anchor count: 1.

116	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	the global budget layer beneath AHEAD, and the RBP price constraint that prevents high-cost providers from offsetting AHEAD risk through commercial rate increases	the global budget layer under state authority, and the RBP price constraint that prevents high-cost providers from offsetting global-budget risk through commercial rate increases	C3-19 STALE (medium). §3.9 national paragraph layers Vermont's architecture under AHEAD, contradicting the same paragraph's withdrawal statement. Sources: https://healthcarereform.vermont.gov/AHEAD-model ; https://www.wcax.com/2026/07/28/vermont-drops-ahead-healthcare-model-shifts-195m-rural-health-fund/ Anchor count: 1.
117	Chapter 4: The Technology Pillar — Data Infrastructure for a Transform Step 3 · raw	It also means that Vermont's AHEAD implementation	It also means that Vermont's global budget implementation	C4-1 STALE (medium). §4.3.2. Sources: https://healthcarereform.vermont.gov/AHEAD-model ; https://www.wcax.com/2026/07/28/vermont-drops-ahead-healthcare-model-shifts-195m-rural-health-fund/ Anchor count: 1.
118	Chapter 4: The Technology Pillar — Data Infrastructure for a Transform Step 3 · raw	The RHT Program, AHEAD requirements, and the AHS-GMCB analytics vendor procurement	The RHT Program, Act 68 requirements, and the AHS-GMCB analytics vendor procurement	C4-5 STALE (medium). §4.8. Sources: https://healthcarereform.vermont.gov/AHEAD-model ; https://www.wcax.com/2026/07/28/vermont-drops-ahead-healthcare-model-shifts-195m-rural-health-fund/ Anchor count: 1.
119	Chapter 4: The Technology Pillar — Data Infrastructure for a Transform Step 3 · raw	you are operating without the most important information your AHEAD performance year will depend on. The hospitals that invest in VHCURES analytics capability now will enter the January 2028 performance year	you are operating without the most important information your first Act 68 global budget year will depend on. The hospitals that invest in VHCURES analytics capability now will enter the FY2028 global budget year	C4-6 STALE (high). §4.10 hospital-executive paragraph addresses an AHEAD performance year Vermont will not have. Sources: https://healthcarereform.vermont.gov/AHEAD-model ; https://www.wcax.com/2026/07/28/vermont-drops-ahead-healthcare-model-shifts-195m-rural-health-fund/ ; https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20As%20Enacted.pdf Anchor count: 1.
120	Chapter 4: The Technology Pillar — Data Infrastructure for a Transform Step 3 · raw	through Medicaid, Blueprint, and AHEAD.	through Medicaid and Blueprint.	C4-8 STALE (low). Key Concepts 'Act 62 of 2025'. Sources: https://healthcarereform.vermont.gov/AHEAD-model ; https://www.wcax.com/2026/07/28/vermont-drops-ahead-healthcare-model-shifts-195m-rural-health-fund/ Anchor count: 1.
121	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	Blueprint participation and AHEAD enhanced primary care payments	Blueprint participation and the Blueprint's enhanced primary care payments	C9-1 STALE (AHEAD as Vermont's live path). PCMH recognition is the gate to the Blueprint's own enhanced payments; AHEAD primary-care payments ended with the July 2026 withdrawal [AHSWEB][VTPUB].
122	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	And AHEAD transition support — helping practices navigate the transition from current Blueprint Medicare payment to Primary Care AHEAD — is	And payment transition support — helping practices absorb the loss of Medicare's Blueprint payments, which ended with the All-Payer Model in 2025 and which state funds are offsetting in 2026 after Vermont withdrew from AHEAD — is	C9-2 STALE x2. Medicare Blueprint payments ended with the All-Payer Model in 2025 and are being offset with state funds in 2026 [BPWEB][VTPUB]; Primary Care AHEAD is no longer Vermont's path [AHSWEB].
123	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	financially accountable under Blueprint and AHEAD.	financially accountable under the Blueprint and their payer contracts.	C9-3 STALE (carry-over item, md ~L4104) [AHSWEB].
124	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	the commercial market, and AHEAD quality accountability.	the commercial market, and value-based contract quality accountability.	C9-4 STALE: "AHEAD quality accountability" as current Vermont measurement regime.
125	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	the full attributed population for AHEAD accountability.	the full attributed population under Act 68 global budgets.	C9-5 STALE x2 in §9.5.1 Step 3 [AHSWEB].

126	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	prerequisite for effective AHEAD population health management.	prerequisite for effective population health management under global budgets.	C9-5 STALE x2 in §9.5.1 Step 3 [AHSWEB].
127	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	clinical leader engagement in AHEAD implementation and Statewide Strategic Plan development. The AHEAD model’s clinical accountability requirements	clinical leader engagement in global budget implementation and Statewide Strategic Plan development. Global budgets’ clinical accountability requirements	C9-9 STALE x2 in §9.7 [AHSWEB].
128	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	The Blueprint provides the framework, AHEAD provides the financial incentive,	The Blueprint provides the framework, Act 68 global budgets provide the financial incentive,	C9-10 STALE: AHEAD as the financial incentive [AHSWEB].
129	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	Blueprint program; DVHA; AHEAD implementation team	Blueprint program; DVHA; AHS	C9-6 STALE: Fig 9.4 vehicle cell names an AHEAD implementation team.
130	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	Clinical leader engagement in AHEAD governance and Statewide Strategic Plan development	Clinical leader engagement in global budget governance and Statewide Strategic Plan development	C9-7 STALE x2: Fig 9.4 last row, AHEAD governance [AHSWEB].
131	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	physician and nursing leadership in AHEAD hospital governance	physician and nursing leadership in hospital global budget governance	C9-7 STALE x2: Fig 9.4 last row, AHEAD governance [AHSWEB].
132	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	the geographic unit for both AHEAD population attribution	the geographic unit for both hospital transformation planning	C9-11 STALE x2: Implications (AHS/GMCB). HSAs are by definition the hospital service areas, so the attribution clause is re-pointed to hospital planning.
133	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	deliver the care and services AHEAD measures.	deliver the care and services the transformation measures.	C9-11 STALE x2: Implications (AHS/GMCB). HSAs are by definition the hospital service areas, so the attribution clause is re-pointed to hospital planning.
134	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	AHEAD financial incentives for deployment	global budget incentives for deployment	C9-12 STALE: Implications (national) [AHSWEB].
135	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	directly affecting global budget levels under AHEAD.	directly affecting global budget levels.	C9-13 STALE: Key Concepts, HCC gap analysis.
136	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	implicit bias training as a condition of Blueprint and AHEAD participation	implicit bias training as a condition of Blueprint participation	C10-4 / C10-5 / C10-6 STALE: Fig 10.3 rows 2-4 list the AHEAD-only EAST Fund and AHEAD participation conditions as live Vermont interventions [AHSWEB].
137	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	cultural competency standards in Blueprint and AHEAD participation requirements	cultural competency standards in Blueprint participation requirements	C10-7 STALE: Fig 10.4 AHEAD participation requirements [AHSWEB].

138	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	Vermont AHEAD: active SRA design question as of early 2026	Vermont global budgets: open SRA design question	C10-14 STALE: Fig 10.8 SRA benchmark [AHSWEB].
139	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	through Comprehensive and Professional Direct Contracting model designs that include social risk stratification. Vermont's AHEAD global budget design	through its health equity benchmark adjustment, which raises benchmarks for ACOs serving underserved beneficiaries. Vermont's Act 68 global budget design	C10-15 WRONG FACT + STALE. There was no "Comprehensive" Direct Contracting; ACO REACH's SRA mechanism is the Health Equity Benchmark Adjustment [REACH]. AHEAD global budget -> Act 68 global budget [AHSWEB].
140	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	active design question in Vermont's AHEAD implementation	open design question in Vermont's global budget methodology	C10-15 WRONG FACT + STALE. There was no "Comprehensive" Direct Contracting; ACO REACH's SRA mechanism is the Health Equity Benchmark Adjustment [REACH]. AHEAD global budget -> Act 68 global budget [AHSWEB].
141	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	HEROI score incorporated into AHEAD quality reporting for Vermont	HEROI score incorporated into GMCB quality reporting for Vermont	C10-16 STALE: Fig 10.9 step 5 [AHSWEB].
142	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	AHEAD's performance measurement framework includes equity metrics — stratified HEDIS measures by race, income, and geography — and Vermont's global budget methodology will incorporate equity performance.	Vermont's global budget methodology should incorporate equity performance — stratified HEDIS measures by race, income, and geography.	C10-17 STALE: tells a Vermont hospital executive that AHEAD's framework governs them; also turns an unsourced "will" into the chapter's own recommendation.
143	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	and \$1.2M in AHEAD cooperative agreement funds		C11-1 STALE: AHEAD cooperative-agreement funds in AHS's current budget [AHSWEB].
144	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	Managing AHEAD Medicaid alignment	Post-AHEAD Medicaid transition (Vermont withdrew July 2026)	C11-3 STALE: Fig 11.1 DVHA current-capacity cell [AHSWEB].
145	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	PMPM cost: below AHEAD total cost of care targets.	PMPM cost: slow the growth trend.	C11-8 STALE: Fig 11.7 target [AHSWEB].
146	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	negotiate with CMS on AHEAD implementation,		C11-9 STALE (and never an Act 68 requirement): §11.8.1.
147	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	(Blueprint, GMCB, AHEAD, commercial payers)	(Blueprint, GMCB, commercial payers)	C11-11 STALE: AHEAD as a current reporting program (§11.15.1) [AHSWEB].
148	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	#NAME?	#ERROR!	C11-12 STALE: Fig 11.9 [AHSWEB].
149	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	for your Medicare and AHEAD-attributed population	for your Medicare-attributed population	C11-13 STALE: Implications (hospital exec) [AHSWEB].
150	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 3 · raw	preparing for AHEAD global budget entry in January 2028	preparing for Act 68 global budgets in FY2028	C12-1 STALE (ledger Part 1): AHEAD global budget entry as live client segment.

151	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 3 · raw	managing simultaneous AHEAD implementation,	managing simultaneous	C12-2 STALE [AHSWEB].
152	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 3 · raw	, and the AHEAD negotiating team.	.	C12-3 STALE: present-tense guiding-coalition member that no longer exists.
153	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 3 · raw	population health improvement AHEAD measures	population health improvement global budgets depend on	C12-5 STALE [AHSWEB].
154	Appendix F — A Reader’s Guide to the Health Transformation Review Plat Step 4a · raw	the AHEAD Model analysis (six states including Vermont)	the AHEAD Model analysis (five participating states, and why Vermont withdrew)	Phase 1, step 4a — book errors surfaced by the promise-delivery audit (2026-10-07). Preface: chapters end with 'Work This Chapter on the Platform' (15 sections); no 'Explore on the Platform' or 'Go Deeper — Academy' sections exist (only 1 of 15 links the Academy). Name the real section; point to the companion course (Introduction: Five Pillars, One Imperative follows the book’s sequence). Appendix F.6: AHEAD participants are MD, CT, HI, RI, downstate NY; Vermont withdrew July 2026 (/ahead-model page).
Statewide Strategic Plan date — 39				
#	Where	Old text	New text	Source / reason
155	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 1 · raw	to be delivered to the legislature by December 2028 and updated every three years thereafter	to be delivered to the legislature by January 15, 2028 and updated every three years thereafter, beginning December 1, 2030	2. Strategic Plan — first statutory descriptions in Ch2 get the exact date
156	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 1 · raw	The plan AHS is required to deliver to the Vermont legislature by December 2028 and update every three years	The plan AHS is required to deliver to the Vermont legislature by January 15, 2028 and update every three years	2. Strategic Plan — first statutory descriptions in Ch2 get the exact date
157	Chapter 1: The Five-Pillar Framework Step 1 · ch_regex	every match of “\bDecember 2028\b”	January 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
158	Chapter 1: The Five-Pillar Framework Step 1 · ch_regex	every match of “\bDec 2028\b”	Jan 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
159	Chapter 2: The Policy Pillar Step 1 · ch_regex	every match of “\bDecember 2028\b”	January 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
160	Chapter 2: The Policy Pillar Step 1 · ch_regex	every match of “\bDec 2028\b”	Jan 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
161	Chapter 4: The Technology Pillar Step 1 · ch_regex	every match of “\bDecember 2028\b”	January 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
162	Chapter 4: The Technology Pillar Step 1 · ch_regex	every match of “\bDec 2028\b”	Jan 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
163	Chapter 6: The Economics Pillar Step 1 · ch_regex	every match of “\bDecember 2028\b”	January 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
164	Chapter 6: The Economics Pillar Step 1 · ch_regex	every match of “\bDec 2028\b”	Jan 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12

165	Chapter 8: The Clinical Pillar Step 1 · ch_regex	every match of “\bDecember 2028\b”	January 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
166	Chapter 8: The Clinical Pillar Step 1 · ch_regex	every match of “\bDec 2028\b”	Jan 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
167	Chapter 9: The Clinical Pillar in Practice Step 1 · ch_regex	every match of “\bDecember 2028\b”	January 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
168	Chapter 9: The Clinical Pillar in Practice Step 1 · ch_regex	every match of “\bDec 2028\b”	Jan 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
169	Chapter 11: The Operations Pillar Step 1 · ch_regex	every match of “\bDecember 2028\b”	January 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
170	Chapter 11: The Operations Pillar Step 1 · ch_regex	every match of “\bDec 2028\b”	Jan 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
171	Chapter 12: Infrastructure for Knowledge Transfer Step 1 · ch_regex	every match of “\bDecember 2028\b”	January 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
172	Chapter 12: Infrastructure for Knowledge Transfer Step 1 · ch_regex	every match of “\bDec 2028\b”	Jan 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
173	Chapter 13: The Future of Healthcare Transformation Step 1 · ch_regex	every match of “December 2028 statutory deadline”	January 15, 2028 statutory deadline	#ERROR!
174	Chapter 13: The Future of Healthcare Transformation Step 1 · ch_regex	every match of “\bDecember 2028\b”	January 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
175	Chapter 13: The Future of Healthcare Transformation Step 1 · ch_regex	every match of “\bDec 2028\b”	Jan 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
176	Chapter 14: Political Sustainability Step 1 · ch_regex	every match of “in November 2028 reports”	in December 2027 reports	Ch14: §14.2.2, Fig 14.1 row, §14.5 (3 x "December 2028"); Fig 14.1 "November 2028 reports" 1 sub
177	Chapter 14: Political Sustainability Step 1 · ch_regex	every match of “\bDecember 2028\b”	January 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
178	Chapter 15: Healthcare Transformation as Portfolio Management Step 1 · ch_regex	every match of “\bDecember 2028\b”	January 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
179	Chapter 15: Healthcare Transformation as Portfolio Management Step 1 · ch_regex	every match of “\bDec 2028\b”	Jan 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
180	Chapter 16: The AHS Restructuring Roadmap Step 1 · ch_regex	every match of “\bDecember 1, 2028\b”	January 15, 2028	Ch16: "December 1, 2028" (§16.1) 1; "Dec 1, 2028" (Fig 16.1) 1; "December 2028" 7 (opener, §16.3.1, Pillar 1, Fig 16.4 row, §16.7, Fig 16.6, §16.9); "Dec 2028" (Fig 16.5) 1 1 sub
181	Chapter 16: The AHS Restructuring Roadmap Step 1 · ch_regex	every match of “\bDec 1, 2028\b”	Jan 15, 2028	Ch16: "December 1, 2028" (§16.1) 1; "Dec 1, 2028" (Fig 16.1) 1; "December 2028" 7 (opener, §16.3.1, Pillar 1, Fig 16.4 row, §16.7, Fig 16.6, §16.9); "Dec 2028" (Fig 16.5) 1 1 sub

182	Chapter 16: The AHS Restructuring Roadmap Step 1 · ch_regex	every match of “\bDecember 2028\b”	January 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
183	Chapter 16: The AHS Restructuring Roadmap Step 1 · ch_regex	every match of “\bDec 2028\b”	Jan 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
184	Conclusion — What Vermont Proves Step 1 · ch_regex	every match of “\bDecember 1, 2028\b”	January 15, 2028	Ch16: "December 1, 2028" (§16.1) 1; "Dec 1, 2028" (Fig 16.1) 1; "December 2028" 7 (opener, §16.3.1, Pillar 1, Fig 16.4 row, §16.7, Fig 16.6, §16.9); "Dec 2028" (Fig 16.5) 1 1 sub
185	Conclusion — What Vermont Proves Step 1 · ch_regex	every match of “\bDecember 2028\b”	January 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
186	Appendix B — Act 68 Statutory Timeline and Milestones Step 1 · row_regex	in row “December 1, 2028 Statewide Strategic Plan delivered to legislature AHS + HCAC In planning — development begins 2026”: December 1, 2028	January 15, 2028	Appendix B, Fig B.1: plan row date, and the update row ("Every 3 years from 2028").
187	Appendix B — Act 68 Statutory Timeline and Milestones Step 1 · raw	Every 3 years from 2028	Every 3 years from Dec 1, 2030	Appendix B, Fig B.1: plan row date, and the update row ("Every 3 years from 2028").
188	Appendix E — Vermont Transformation Scorecard Step 1 · ch_regex	every match of “\bDecember 2028\b”	January 2028	2. Strategic Plan — every remaining occurrence in Chapters 1–12
189	Appendix G — Adapting and Stress-Testing the Framework Step 1 · raw	Strategic Plan (due December 2028).	Strategic Plan (due January 15, 2028).	Appendix G: G.1 Act 68 row "(due December 2028)"; G.3.2 "(due December 2028 — itself after the highest-risk window closes) will need to address retroactively" — Jan 15, 2028 falls INSIDE the FY2027–2028 window, so the clause's logic is restated, not just the date.
190	Appendix G — Adapting and Stress-Testing the Framework Step 1 · raw	(due December 2028 — itself after the highest-risk window closes) will need to address retroactively	(due January 15, 2028 — more than a year into the highest-risk window) will need to address retroactively	Appendix G: G.1 Act 68 row "(due December 2028)"; G.3.2 "(due December 2028 — itself after the highest-risk window closes) will need to address retroactively" — Jan 15, 2028 falls INSIDE the FY2027–2028 window, so the clause's logic is restated, not just the date.
191	Chapter 13: The Future of Healthcare Transformation — 2026, What Verno Step 1 · raw	FY2027: RBP implemented; commercial rates decline toward benchmark;	FY2027–FY2028: RBP rule set in 2027 and prices in effect from FY2028; commercial rates decline toward benchmark;	#ERROR!
192	Chapter 14: Political Sustainability — Protecting Transformation Across Step 2b · raw	Mandatory participation, statutory deadlines, and federal-state agreements survive changes of government; strategic plans and voluntary commitments generally do not.	Mandatory participation and statutory deadlines survive changes of government; federal-state agreements last only while their economics hold, and strategic plans and voluntary commitments generally do not.	§14.7 — federal agreements can no longer be listed among the durable protections.
193	Chapter 1: The Five-Pillar Framework and the Execution Sequence Step 3 · raw	A delay in that capability’s deployment does not merely delay one program: it delays the financial management capability for Act 68 global budgets, the equity measurement capability for the Statewide Strategic Plan, and the risk stratification capability for Blueprint’s CCBHC expansion. Three dependencies converge on this single Technology pillar investment.	A delay in that capability’s deployment holds up all three programs that converge on it (§1.6).	C1-3 REPETITION (low). §1.14.1 restates §1.6 point 2 ('a delay there does not delay one program but three ...') clause for clause. §1.17's AHS paragraph is a role- specific application and stays. Also removes the second 'Blueprint's CCBHC' misattribution. Anchor count: 1.
Reference-based pricing timing — 33				
#	Where	Old text	New text	Source / reason

194	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 1 · raw	RBP is not a theoretical target; it is a statutory mandate effective FY2027,	RBP is not a theoretical target; it is a statutory mandate — Act 68 requires it no later than hospital FY2027, and GMCB sets the methodology by rule in 2027 with prices taking effect in hospital FY2028 —	5. RBP — reconcile the statutory deadline with GMCB's schedule where Ch2 explains it
195	Chapter 1: The Five-Pillar Framework and the Execution Sequence Step 1 · raw	The GMCB’s RBP methodology for FY2027 is being refined	The GMCB’s RBP methodology, to be set by rule in 2027 for prices effective in hospital FY2028, is being refined	5. Chapter 1
196	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 1 · raw	Premium reduction data from RBP (available FY2027-2028)	Premium reduction data from RBP (available from FY2028)	5. Chapter 12
197	Chapter 13: The Future of Healthcare Transformation — 2026, What Vermo Step 1 · raw	VT: RBP implementing (FY2027); global budgets mandated (FY2028).	VT: RBP rule-making (2027; prices from FY2028); global budgets mandated (FY2028).	5. Chapter 13 status table (missed by the Ch13 analyst)
198	Chapter 13: The Future of Healthcare Transformation — 2026, What Vermo Step 1 · raw	FY2027: RBP implemented but contested —	FY2027–FY2028: RBP rule contested —	Ch13 §13.5.2 partial scenario
199	Chapter 13: The Future of Healthcare Transformation — 2026, What Vermo Step 1 · raw	FY2027 implementation; first evidence FY2028	Rule set 2027; prices effective FY2028; first evidence FY2029	Ch13 Fig 13.4 row 1, Vermont timeline cell
200	Chapter 14: Political Sustainability — Protecting Transformation Across Step 1 · raw	RBP is mandatory from FY2027, but the	RBP must begin by FY2027 under Act 68 — GMCB has said FY2028 is the earliest prices can take effect — but the	Ch14 §14.2.2 "RBP is mandatory from FY2027" ("methodology" that follows is an italic run, so the anchor stops at the run boundary.)
201	Chapter 15: Healthcare Transformation as Portfolio Management — Applyi Step 1 · raw	Methodology design → FY2027	Methodology design → FY2028 rates in effect	Ch15 Fig 15.3 RBP implementation stage gate, and Fig 15.4 Economics deliverable
202	Chapter 15: Healthcare Transformation as Portfolio Management — Applyi Step 1 · raw	RBP rates in effect (FY2027)	RBP rates in effect (FY2028)	Ch15 Fig 15.3 RBP implementation stage gate, and Fig 15.4 Economics deliverable
203	Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework fo Step 1 · raw	RBP maximum rates in effect for commercial payers	RBP implementation begins (statutory deadline); maximum rates set by GMCB rule in 2027, in effect FY2028	Ch16 Fig 16.4 RBP row milestone
204	Appendix B — Act 68 Statutory Timeline and Milestones Step 1 · raw	Reference-based pricing mandatory for commercial payers — maximum rates set by GMCB	Reference-based pricing due (statutory deadline); maximum rates set by GMCB rule in 2027, in effect FY2028	App B Fig B.1 RBP row requirement
205	Appendix G — Adapting and Stress-Testing the Framework Step 1 · raw	commercial RBP becomes mandatory in FY2027 (October 2026),	commercial RBP must begin by FY2027 (October 2026), with maximum rates in effect from FY2028,	App G "THE GAP" callout
206	Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework fo Step 2b · tbl_regex	FY2028 (Jan 2028) Act 68 hospital global budgets take effect (FY2028) AHS + DVHA + GMCB + CMS Nine-year model; ongoing federal coordination	(table row deleted)	Fig 16.4: delete the AHEAD-remnant row "FY2028 (Jan 2028) Act 68 hospital global budgets take

207	Conclusion — What Vermont Proves, and What Remains to Be Proven Step 2b · raw	January 2028 is 21 months away.	FY2028 begins in October 2027, 18 months away.	--- Conclusion
208	Conclusion — What Vermont Proves, and What Remains to Be Proven Step 2b · raw	the window is January 2028. Everything else	the window closes when FY2028 begins in October 2027. Everything else	--- Conclusion
209	Conclusion — What Vermont Proves, and What Remains to Be Proven Step 2b · raw	Will the analytics vendor be operational before January 2028?	Will the analytics vendor be operational before FY2028?	--- Conclusion
210	Chapter 15: Healthcare Transformation as Portfolio Management — Applyi Step 2b · tbl_regex	in row “Clinical HEROI equity-scoring methodology Analytical project High AHS + Health Equity Studio In development → iterative refinement through FY2028”: (?s)((?:?!).)*?RHT Program deployment.*?)(.??)((?:?!).)*?Social-risk-adjustment methodology.*?)	\\1\\3\\2	Ch15 Fig 15.3: the two rows added to restore 19 components were appended after Operations, breaking the pillar grouping §15.7 tells the reader to rely on ("Read Figure 15.3 by its first column ... the components are already grouped by pillar"). Move each into its pillar block.
211	Chapter 15: Healthcare Transformation as Portfolio Management — Applyi Step 2b · tbl_regex	in row “Clinical HEROI equity-scoring methodology Analytical project High AHS + Health Equity Studio In development → iterative refinement through FY2028”: (?s)((?:?!).)*?CoCM statewide deployment.*?)(.??)((?:?!).)*?HEROI equity-scoring methodology.*?)	\\1\\3\\2	Ch15 Fig 15.3: the two rows added to restore 19 components were appended after Operations, breaking the pillar grouping §15.7 tells the reader to rely on ("Read Figure 15.3 by its first column ... the components are already grouped by pillar"). Move each into its pillar block.
212	Chapter 5: The Technology Pillar in Practice — FHIR, AI Governance, an Step 2c · raw	the analytics vendor gap that must close before January 2028.	the analytics vendor gap that must close before global budgets take effect in FY2028.	C5-1 / C5-2 / C5-3 STALE DEADLINE. "January 2028" is the AHEAD 2.0 Medicare global-budget start for Cohort 2 [GMCB26, Table 4], i.e. the deadline Vermont left when it withdrew in July 2026 [AHSWEB]. The deadline that survives is Act 68's FY2028 global-budget start, which Ch6 §6.8.2 already uses for this exact analytics gap ("This gap must be closed before global budgets take effect in FY2028"). Anchors: count 1 each, single run.
213	Chapter 5: The Technology Pillar in Practice — FHIR, AI Governance, an Step 2c · raw	Vermont’s January 2028 deadline simply compresses	Vermont’s FY2028 global budget deadline simply compresses	C5-1 / C5-2 / C5-3 STALE DEADLINE. "January 2028" is the AHEAD 2.0 Medicare global-budget start for Cohort 2 [GMCB26, Table 4], i.e. the deadline Vermont left when it withdrew in July 2026 [AHSWEB]. The deadline that survives is Act 68's FY2028 global-budget start, which Ch6 §6.8.2 already uses for this exact analytics gap ("This gap must be closed before global budgets take effect in FY2028"). Anchors: count 1 each, single run.
214	Chapter 5: The Technology Pillar in Practice — FHIR, AI Governance, an Step 2c · raw	If your FHIR infrastructure is not production-ready before January 2028,	If your FHIR infrastructure is not production-ready before global budgets take effect in FY2028,	C5-1 / C5-2 / C5-3 STALE DEADLINE. "January 2028" is the AHEAD 2.0 Medicare global-budget start for Cohort 2 [GMCB26, Table 4], i.e. the deadline Vermont left when it withdrew in July 2026 [AHSWEB]. The deadline that survives is Act 68's FY2028 global-budget start, which Ch6 §6.8.2 already uses for this exact analytics gap ("This gap must be closed before global budgets take effect in FY2028"). Anchors: count 1 each, single run.

215	Chapter 6: The Economics Pillar — Global Budgets, Reference-Based Pricing Step 2c · raw	Targeting implementation FY27; first negative	Methodology set by rule in 2027, effective hospital FY2028; first negative	C6-2 / C6-3 / C6-4 INTERNAL CONTRADICTION + STALE. §6.3.1 ("by FY2027"), Fig 6.3 Vermont row ("(FY2027)", "Targeting implementation FY27") disagree with the chapter's own Fig 6.4 ("Final RBP methodology released March 2027 for FY28 hospital budget guidance") and with GMCB: "methodology set by rule in 2027, effective HFY28" [GMCB26 slide 12]; Act 68's FY2027 is the statutory outer date but "the earliest reference-based pricing can be effectuated is for hospital fiscal year 2028" [GMCB26; ACT68S].
216	Chapter 6: The Economics Pillar — Global Budgets, Reference-Based Pricing Step 2c · raw	RBP maximum rates in effect for commercial payers; Act 68 hospital global budgets take effect FY2028	Final RBP methodology released March 2027 for FY2028 rates; Act 68 hospital global budgets take effect FY2028	C6-8 Fig 6.11 Transition (FY2027) row says RBP rates are "in effect" in FY2027; contradicts Fig 6.4 and GMCB (effective HFY28) [GMCB26].
217	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · raw	FY2027: systemwide savings	FY2028: systemwide savings	C7-6 / C7-7 Fig 7.2 RBP row dates savings/effect to FY2027; GMCB: effective HFY28 [GMCB26]. Same correction as C6-2..C6-4.
218	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · raw	Vermont mandatory RBP FY2027;	Vermont mandatory RBP effective FY2028;	C7-6 / C7-7 Fig 7.2 RBP row dates savings/effect to FY2027; GMCB: effective HFY28 [GMCB26]. Same correction as C6-2..C6-4.
219	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · raw	or 250% (GMCB’s phased approach)	or 250% (an upper scenario GMCB modeled in its December 2024 RBP report)	C7-9 UNSUPPORTED ATTRIBUTION + contradicts Ch6 §6.10 (ceiling "to be set by GMCB rule in March 2027"). 250% is one of four sensitivity scenarios (150/180/200/250%) in GMCB's Dec 2024 RBP report [GMCB24]; GMCB's Feb 2026 update names no 250% target [GMCB26]. (200% as Oliver Wyman's recommendation IS supported: [GMCB24] "levels recommended by Oliver Wyman ... at 200%".) NOTE: the Hospital Stress Test preset label "250% ... GMCB phased benchmark" (HospitalFinancialScorecard.tsx:115) carries the same error -- platform fix.
220	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 3 · raw	First mandatory all-payer price cap in modern Vermont history; GMCB sets the maximum by rule (Oliver Wyman benchmark: 200% of Medicare or less)	Statutory deadline for the first mandatory all-payer price cap in modern Vermont history; GMCB sets the maximum by rule in 2027, effective hospital FY2028 (Oliver Wyman benchmark: 200% of Medicare or less)	C2-1 STALE (medium). Fig T.1 FY2027 row presents the cap as in effect in FY2027 (fixed fact #5). The Milestone cell ('RBP mandatory for all Vermont hospitals') states the statutory deadline and stays. Sources: https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20As%20Enacted.pdf ; https://legislature.vermont.gov/Documents/2026/Workgroups/House%20Health%20Care/Reports%20and%20Resources/W~Green%20Mountain%20Care%20Board~Act%2068%20Update~2-17-2026.pdf Anchor count: 1.
221	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 3 · row_regex	in row “Jan 2028 Act 68 hospital global budgets take effect (FY2028) Economics / Technology Updated per CMS Sept 2025; Vermont hospitals begin Act 68 global-budget accountability (FY2028)”: (?s)\A.*\Z	(formatting change only)	C2-2 CONTRADICTION (medium). Fig T.1 duplicates its own FY2028 (Oct 2027) row with the wrong month (FY2028 begins Oct 2027) and an AHEAD vestige ('Updated per CMS Sept 2025' is AHEAD Cohort-2 timing). Act 68 Sec. 2: global budgets 'not later than hospital fiscal year 2028'. row_regex deletes the whole <w:tr>. Sources: https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20As%20Enacted.pdf ; https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20Act%20Summary.pdf Anchor count: 1.
222	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 3 · raw	Methodology set by rule during FY2026; maximum allowable payments mandatory from FY2027.	Methodology set by rule in 2027; maximum allowable payments effective from hospital FY2028 (statutory deadline: FY2027).	C2-4 STALE (high). Fig 2.4 Vermont row was set to the old FY2026/FY2027 timeline by the 2026-09 audit (its M2); step-1 fact #5 overtakes it. §2.11 already uses the corrected timeline. Sources: https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20As%20Enacted.pdf ; https://legislature.vermont.gov/Documents/2026/Workgroups/House%20Health%20Care/Reports%20and%20Resources/W~Green%20Mountain%20Care%20Board~Act%2068%20Update~2-17-2026.pdf Anchor count: 1.

223	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 3 · raw	RBP vendor selection and methodology finalized; rulemaking completed	Reference-based prices take effect (hospital FY2028) — maximum amounts hospitals can charge commercial payers, set by GMCB rule	C2-5a STALE (medium). APPLY BEFORE C2-5b (C2-5b's replacement contains this op's find). The 'Reform Cascade Timeline' box listed RBP 'implemented' before rulemaking; rule is 2027, prices FY2028. Sources: https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20As%20Enacted.pdf ; https://legislature.vermont.gov/Documents/2026/Workgroups/House%20Health%20Care/Reports%20and%20Resources/W~Green%20Mountain%20Care%20Board~Act%2068%20Update~2-17-2026.pdf Anchor count: 1.
224	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 3 · raw	Reference-based pricing for hospital services implemented — maximum amounts hospitals can charge commercial payers, set by GMCB rule	RBP vendor selection and methodology finalized; rulemaking completed (2027)	C2-5b STALE (medium). Pair with C2-5a: swaps the two bullets into true order. Sources: https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20As%20Enacted.pdf ; https://legislature.vermont.gov/Documents/2026/Workgroups/House%20Health%20Care/Reports%20and%20Resources/W~Green%20Mountain%20Care%20Board~Act%2068%20Update~2-17-2026.pdf Anchor count: 1.
225	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	operating under prospective hospital global budgets beginning January 2028	operating under prospective hospital global budgets beginning in hospital FY2028	C3-18 STALE (medium). §3.9: January 2028 is AHEAD Cohort-2 timing; Act 68 is FY2028 (Oct 2027), as the next paragraph says. Sources: https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20As%20Enacted.pdf ; https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20Act%20Summary.pdf Anchor count: 1.
226	Chapter 4: The Technology Pillar — Data Infrastructure for a Transform Step 3 · raw	must be operational before January 2028:	must be operational before Act 68 global budgets take effect in FY2028:	C4-7 STALE (medium). §4.10 AHS paragraph; January 2028 is AHEAD Cohort-2 timing. Sources: https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20As%20Enacted.pdf ; https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20Act%20Summary.pdf Anchor count: 1.
Medicaid work requirements — 7				
#	Where	Old text	New text	Source / reason
227	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 1 · raw	for Medicaid expansion enrollees beginning late 2026.	for Medicaid expansion enrollees beginning January 1, 2027.	4. Work requirements — Chapter 3
228	Chapter 13: The Future of Healthcare Transformation — 2026, What Vermo Step 1 · raw	on Medicaid-expansion enrollees beginning in late 2026, requires	on Medicaid-expansion enrollees beginning January 1, 2027, requires	#ERROR!
229	Chapter 13: The Future of Healthcare Transformation — 2026, What Vermo Step 1 · raw	pressure hospital finances beginning in 2026–2027; the work-requirement implementation, starting in late 2026,	pressure hospital finances beginning in 2027; the work-requirement implementation, starting in January 2027,	#ERROR!
230	Chapter 13: The Future of Healthcare Transformation — 2026, What Vermo Step 1 · raw	Medicaid work requirements (late 2026);	Medicaid work requirements (January 2027);	#ERROR!
231	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	H.R. 1 (2025) explicitly authorizes work requirements for Medicaid expansion enrollees, resolving the prior legal uncertainty in favor of state authority. Vermont has not implemented work requirements and has not signaled intent to do so.	H.R. 1 (2025) requires work requirements for Medicaid expansion enrollees beginning January 1, 2027. As an expansion state, Vermont must apply them from that date; DVHA has begun notifying members.	C3-7 WRONG FACT (high). §3.3.2. H.R. 1 makes the requirement mandatory for expansion states (fixed fact #4); 'Vermont has not signaled intent' is wrong (DVHA screening tool, StayCoveredVT.com). Sources: https://vtlawhelp.org/2027-medicaid-changes ; https://dvha.vermont.gov/medicaid-work-requirements-screening-tool Anchor count: 1.

232	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	Vermont has not expressed intent to implement work requirements; federal law now clearly authorizes them.	Mandatory for Vermont’s expansion adults (ages 19–64) from January 1, 2027; DVHA has begun notifying members.	C3-9 WRONG FACT (high). Fig 3.4 Vermont cell; the following sentence ('Coverage implications ... require monitoring.') stays. Sources: https://vtlawhelp.org/2027-medicaid-changes ; https://dvha.vermont.gov/medicaid-work-requirements-screening-tool Anchor count: 1.
233	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · row_regex	in row “Eligibility redetermination every 6 months (vs. annual) 2026 Increases administrative burden; produces coverage churn as enrollees who remain eligible lose coverage due to administrative failures;”: 2026	January 2027	C3-10 STALE (medium). Fig 3.4 six-month redetermination row; fixed fact #4. The row contains exactly one '>2026</w:t>' (verified). Sources: https://vtlawhelp.org/2027-medicaid-changes ; https://dvha.vermont.gov/medicaid-work-requirements-screening-tool Anchor count: 1.
CCBHCs — 16				
#	Where	Old text	New text	Source / reason
234	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	Vermont’s RHT Program application plans to certify five additional CCBHC entities by July 2026	Vermont’s RHT Program application planned to certify five additional CCBHC entities by July 2026	C8-29 / C8-30 / C8-31 OVERTAKEN BY EVENTS: four of the five planned 2026 CCBHCs (NKHS, Howard, NCSS, HCRS -- not Washington County MHS) were certified July 1, 2026, bringing Vermont to six [DMH; VCP]. "As of 2025 ... two active" is CORRECT (Clara Martin Center, Rutland MHS -- 2025 cohort [DMH]) and is kept.
235	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	and Washington County Mental Health Services (Barre).	and Washington County Mental Health Services (Barre). The Department of Mental Health certified four of them — all but Washington County — on July 1, 2026, bringing Vermont to six CCBHCs.	C8-29 / C8-30 / C8-31 OVERTAKEN BY EVENTS: four of the five planned 2026 CCBHCs (NKHS, Howard, NCSS, HCRS -- not Washington County MHS) were certified July 1, 2026, bringing Vermont to six [DMH; VCP]. "As of 2025 ... two active" is CORRECT (Clara Martin Center, Rutland MHS -- 2025 cohort [DMH]) and is kept.
236	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	Vermont’s CCBHC expansion target: 5 new certified entities by July 2026, the first tranche toward coverage of every Hospital Service Area.	Vermont’s CCBHC expansion: 4 new entities certified July 1, 2026 (of 5 planned), bringing the state to six — the first tranche toward coverage of every Hospital Service Area.	C8-29 / C8-30 / C8-31 OVERTAKEN BY EVENTS: four of the five planned 2026 CCBHCs (NKHS, Howard, NCSS, HCRS -- not Washington County MHS) were certified July 1, 2026, bringing Vermont to six [DMH; VCP]. "As of 2025 ... two active" is CORRECT (Clara Martin Center, Rutland MHS -- 2025 cohort [DMH]) and is kept.
237	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	2026 (5 new CCBHCs) through 2028	2026 (4 new CCBHCs certified July 1) through 2028	C8-29 / C8-30 / C8-31 OVERTAKEN BY EVENTS: four of the five planned 2026 CCBHCs (NKHS, Howard, NCSS, HCRS -- not Washington County MHS) were certified July 1, 2026, bringing Vermont to six [DMH; VCP]. "As of 2025 ... two active" is CORRECT (Clara Martin Center, Rutland MHS -- 2025 cohort [DMH]) and is kept.
238	Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework fo Step 2d · raw	CCBHC network development (5 new entities by July 2026	CCBHC network development (4 of 5 planned new entities certified July 1, 2026	Phase 1, step 2d — rule 13 sweep after step 2c's Ch8 CCBHC correction: the same "5 new CCBHCs by July 2026, on track" claim in Ch16 (prose + Fig 16.x row) and Appendix B. Fact: four agencies (NKHS, Howard Center, NCSS, HCRS) certified July 1, 2026, of five planned (WCAX 2026-07-24; VermontBiz "Four more agencies..."; DMH).
239	Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework fo Step 2d · raw	CCBHC certification for 5 new entities (July 2026 target)	CCBHC certification for new entities (4 of 5 planned certified July 1, 2026)	Phase 1, step 2d — rule 13 sweep after step 2c's Ch8 CCBHC correction: the same "5 new CCBHCs by July 2026, on track" claim in Ch16 (prose + Fig 16.x row) and Appendix B. Fact: four agencies (NKHS, Howard Center, NCSS, HCRS) certified July 1, 2026, of five planned (WCAX 2026-07-24; VermontBiz "Four more agencies..."; DMH).
240	Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework fo Step 2d · raw	On track per RHT plan	Four certified July 2026; fifth pending	Phase 1, step 2d — rule 13 sweep after step 2c's Ch8 CCBHC correction: the same "5 new CCBHCs by July 2026, on track" claim in Ch16 (prose + Fig 16.x row) and Appendix B. Fact: four agencies (NKHS, Howard Center, NCSS, HCRS) certified July 1, 2026, of five planned (WCAX 2026-07-24; VermontBiz "Four more agencies..."; DMH).

241	Appendix B — Act 68 Statutory Timeline and Milestones Step 2d · raw	Five new CCBHC entities certified (RHT target)	New CCBHC entities certified (RHT target: five)	Phase 1, step 2d — rule 13 sweep after step 2c's Ch8 CCBHC correction: the same "5 new CCBHCs by July 2026, on track" claim in Ch16 (prose + Fig 16.x row) and Appendix B. Fact: four agencies (NKHS, Howard Center, NCSS, HCRS) certified July 1, 2026, of five planned (WCAX 2026-07-24; VermontBiz "Four more agencies..."; DMH).
242	Appendix B — Act 68 Statutory Timeline and Milestones Step 2d · raw	On track per RHT schedule	Four certified July 1, 2026; fifth pending	Phase 1, step 2d — rule 13 sweep after step 2c's Ch8 CCBHC correction: the same "5 new CCBHCs by July 2026, on track" claim in Ch16 (prose + Fig 16.x row) and Appendix B. Fact: four agencies (NKHS, Howard Center, NCSS, HCRS) certified July 1, 2026, of five planned (WCAX 2026-07-24; VermontBiz "Four more agencies..."; DMH).
243	Chapter 1: The Five-Pillar Framework and the Execution Sequence Step 3 · raw	CCBHC expansion (5 entities by July 2026)	CCBHC expansion (4 of 5 planned new entities certified July 1, 2026)	C1-2 STALE (medium). Fig 1.5 Clinical row. Same wording as the committed step-2d fix (Ch16/App B). Sources: https://mentalhealth.vermont.gov/ccbhc ; https://vermontbiz.com (Vermont Care Partners, 11 Aug 2026: four agencies certified July 1, 2026) Anchor count: 1.
244	Chapter 1: The Five-Pillar Framework and the Execution Sequence Step 3 · raw	risk stratification for Blueprint's CCBHC expansion at once	risk stratification for CCBHC expansion at once	C1-4 WRONG FACT (low). §1.6: CCBHCs are certified by the Department of Mental Health; they are not a Blueprint for Health program. Sources: https://mentalhealth.vermont.gov/ccbhc Anchor count: 1.
245	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 3 · raw	CCBHC expansion to certify five additional community behavioral health clinic entities by July 2026.	CCBHC expansion to certify five additional community behavioral health clinic entities, four of which were certified on July 1, 2026.	C2-6 STALE (medium). §2.8; same fact as the committed step-2d fix. Sources: https://mentalhealth.vermont.gov/ccbhc ; https://vermontbiz.com (Vermont Care Partners, 11 Aug 2026: four agencies certified July 1, 2026) Anchor count: 1.
246	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	Vermont: 5 planned CCBHCs; enhanced Medicaid FMAP funding for 8 years	Vermont: 6 certified CCBHCs (4 added July 1, 2026); enhanced Medicaid FMAP for the 4-year demonstration	C9-8 STALE count + WRONG FACT duration (Fig 9.5). Six CCBHCs certified, four of them July 1, 2026 [DMH][VCP] — Ch8 §8.3 already says "bringing the state to six". The 2024 demonstration cohort runs four years, not eight [CRS].
247	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	CCBHC designation for Northeast Kingdom Human Services (planned July 2026)	CCBHC certification of Northeast Kingdom Human Services (certified July 1, 2026)	C10-13 STALE: NKHS was certified July 1, 2026 [DMH][VCP].
248	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	2026 (CCBHC designation)	July 2026 (CCBHC certified)	C10-13 STALE: NKHS was certified July 1, 2026 [DMH][VCP].
249	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 3 · raw	the RHRC engagement, and the CCBHC certifications	the hospital transformation plans, and the CCBHC certifications	C12-6 CONTRADICTION: a future PMO cannot manage the RHRC engagement, which concluded October 2025 (Ch11).
Repetition removed — 11				
#	Where	Old text	New text	Source / reason

250	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 2 · del_para	Every Vermont hospital should conduct a retrospective HCC gap analysis for their attributed Medicare population before Act 68 global budgets take effect in FY2028.	(paragraph deleted)	Phase 1, step 2a — Chapter 11 and Chapter 12 repetition (audit_chapter.py criterion 2), plus one stale AHEAD-participation heading in the same Ch11 section. Ch11 §11.12.1: the three one-line paragraphs under the heading restate, nearly verbatim, the "VERMONT IN PRACTICE — HCC Coding" callout that closes the section (its "Recommended action" and "Getting HCC coding right..." sentences). The callout is the fuller statement, so the three lead lines go. Heading "Pre-AHEAD Action" presents AHEAD as Vermont's next step; Vermont withdrew from AHEAD. The section's own text anchors on Act 68 global budgets (FY2028). Ch11 PPS vs CAH "Operating profit per adjusted discharge" rows: legitimate — two rows of one table, one per hospital type. Left. Ch12: the italic line under the chapter opening ("Healthcare transformation fails more often ... This chapter covers the HTR platform tools, Advisory service lines...") is a stray teaser no other chapter carries; its first sentence is repeated verbatim in the body.
251	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 2 · del_para	An organization that has under-coded its population's chronic conditions will receive a lower global budget than its actual cost burden warrants.	(paragraph deleted)	Phase 1, step 2a — Chapter 11 and Chapter 12 repetition (audit_chapter.py criterion 2), plus one stale AHEAD-participation heading in the same Ch11 section. Ch11 §11.12.1: the three one-line paragraphs under the heading restate, nearly verbatim, the "VERMONT IN PRACTICE — HCC Coding" callout that closes the section (its "Recommended action" and "Getting HCC coding right..." sentences). The callout is the fuller statement, so the three lead lines go. Heading "Pre-AHEAD Action" presents AHEAD as Vermont's next step; Vermont withdrew from AHEAD. The section's own text anchors on Act 68 global budgets (FY2028). Ch11 PPS vs CAH "Operating profit per adjusted discharge" rows: legitimate — two rows of one table, one per hospital type. Left. Ch12: the italic line under the chapter opening ("Healthcare transformation fails more often ... This chapter covers the HTR platform tools, Advisory service lines...") is a stray teaser no other chapter carries; its first sentence is repeated verbatim in the body.
252	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 2 · del_para	Getting HCC coding right before global budgets take effect is foundational financial preparation.	(paragraph deleted)	Phase 1, step 2a — Chapter 11 and Chapter 12 repetition (audit_chapter.py criterion 2), plus one stale AHEAD-participation heading in the same Ch11 section. Ch11 §11.12.1: the three one-line paragraphs under the heading restate, nearly verbatim, the "VERMONT IN PRACTICE — HCC Coding" callout that closes the section (its "Recommended action" and "Getting HCC coding right..." sentences). The callout is the fuller statement, so the three lead lines go. Heading "Pre-AHEAD Action" presents AHEAD as Vermont's next step; Vermont withdrew from AHEAD. The section's own text anchors on Act 68 global budgets (FY2028). Ch11 PPS vs CAH "Operating profit per adjusted discharge" rows: legitimate — two rows of one table, one per hospital type. Left. Ch12: the italic line under the chapter opening ("Healthcare transformation fails more often ... This chapter covers the HTR platform tools, Advisory service lines...") is a stray teaser no other chapter carries; its first sentence is repeated verbatim in the body.
253	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 2 · del_para	Healthcare transformation fails more often because of change management failures than analytical failures. This chapter covers the HTR platform tools, Advisory service lines, and the implementation toolkit.	(paragraph deleted)	Phase 1, step 2a — Chapter 11 and Chapter 12 repetition (audit_chapter.py criterion 2), plus one stale AHEAD-participation heading in the same Ch11 section. Ch11 §11.12.1: the three one-line paragraphs under the heading restate, nearly verbatim, the "VERMONT IN PRACTICE — HCC Coding" callout that closes the section (its "Recommended action" and "Getting HCC coding right..." sentences). The callout is the fuller statement, so the three lead lines go. Heading "Pre-AHEAD Action" presents AHEAD as Vermont's next step; Vermont withdrew from AHEAD. The section's own text anchors on Act 68 global budgets (FY2028). Ch11 PPS vs CAH "Operating profit per adjusted discharge" rows: legitimate — two rows of one table, one per hospital type. Left. Ch12: the italic line under the chapter opening ("Healthcare transformation fails more

254	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · del_para	A program preventing 50 hospitalizations in 10,000 Medicare beneficiaries at \$15,000 each generates \$750,000 in avoided cost.	(paragraph deleted)	C7-3 / C7-4 / C7-5 DUPLICATION (audit_ch07 Other C): three plain body paragraphs (no shading, border, indent or callout style in the XML) restate the 50 hospitalizations / \$15,000 / \$750,000 / \$200,000 / 275% / FFS-discourages- prevention content of the two paragraphs that immediately follow them.
255	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · del_para	If the program costs \$200,000 to operate, the ROI is 275%.	(paragraph deleted)	C7-3 / C7-4 / C7-5 DUPLICATION (audit_ch07 Other C): three plain body paragraphs (no shading, border, indent or callout style in the XML) restate the 50 hospitalizations / \$15,000 / \$750,000 / \$200,000 / 275% / FFS-discourages- prevention content of the two paragraphs that immediately follow them.
256	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · del_para	Under fee-for-service, the prevented hospitalization is lost revenue — which is why FFS systematically discourages prevention.	(paragraph deleted)	C7-3 / C7-4 / C7-5 DUPLICATION (audit_ch07 Other C): three plain body paragraphs (no shading, border, indent or callout style in the XML) restate the 50 hospitalizations / \$15,000 / \$750,000 / \$200,000 / 275% / FFS-discourages- prevention content of the two paragraphs that immediately follow them.
257	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 5 · del_para	The first half of this chapter established the Vermont equity landscape — the geographic disparities that concentrate in the Northeast Kingdom, the racial and demographic disparities in coverage and access, the SDOH burden that compounds both, and the GLP-1 access crisis that represents the most consequential new equity issue in American healthcare. This chapter develops the analytical and operational toolkit for actually closing those gaps.	(paragraph deleted)	S5-6a Ch10 §10.10 opening paragraph restates the §10.9 bridge ("The remainder of this chapter turns that equity landscape into an operational toolkit") as a recap of the chapter's first half, and calls the same chapter "this chapter" as if it were a new one. §10.10's second paragraph (measurement vs. program, Acts 167/68 accountability) is unique -> kept.
258	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 5 · del_para	These sections develop the operational mechanics of healthcare transformation — the specific processes, metrics, and management disciplines that determine whether value-based care contracts produce the results they are designed for. They fall into four operational domains: revenue cycle transformation for VBC, HCC coding and risk adjustment excellence, credentialing efficiency, and the workforce operations framework.	(paragraph deleted)	S5-6b Ch11 §11.10 second paragraph restates the §11.9 bridge and the §11.9 heading (revenue cycle / HCC / credentialing ...) as a roadmap -- and its "four operational domains" list omits administrative simplification, which §11.15 covers. §11.10's first paragraph (the "operations are the substrate" thesis) is unique -> kept.
259	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 5 · del_para	12.2.2 Four Components of the HTR Platform	(paragraph deleted)	S5-6c / S5-6d Ch12: heading "12.2.2 Four Components of the HTR Platform" restates heading "12.2.1 The HTR Platform — Four Components", and its lead-in sentence restates both. Remove the duplicate heading and lead-in; Figure 12.1 then follows the 12.2.1 list as the table that summarises it (legitimate recurrence). The 12.2.1 list stays: it alone carries the platform URLs, the Convergence Newsletter and the advisory e-mail. (Heading bookmark _lq2plq41pd5s has no references.) Leaves 12.2.1 as a lone subsection under 12.2 -- flagged for the author, not renumbered here.
260	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 5 · del_para	The transformation implementation platform has four integrated components, each designed for a different kind of engagement with the transformation agenda:	(paragraph deleted)	S5-6c / S5-6d Ch12: heading "12.2.2 Four Components of the HTR Platform" restates heading "12.2.1 The HTR Platform — Four Components", and its lead-in sentence restates both. Remove the duplicate heading and lead-in; Figure 12.1 then follows the 12.2.1 list as the table that summarises it (legitimate recurrence). The 12.2.1 list stays: it alone carries the platform URLs, the Convergence Newsletter and the advisory e-mail. (Heading bookmark _lq2plq41pd5s has no references.) Leaves 12.2.1 as a lone subsection under 12.2 -- flagged for the author, not renumbered here.

Other factual / consistency fixes — 126				
#	Where	Old text	New text	Source / reason
261	Chapter 13: The Future of Healthcare Transformation — 2026, What Vermo Step 2b · raw	the most ambitious federal payment reform currently operating, with Vermont’s experience as its clearest warning so far.	the most ambitious federal payment reform currently operating.	Ch13 §13.3.5 — the paragraph makes the "Vermont as warning" point twice in two sentences.
262	Chapter 14: Political Sustainability — Protecting Transformation Acros Step 2b · raw	The political sustainability of Vermont’s reform ultimately rests on one thing:	At the organizational level, the political sustainability of Vermont’s reform rests on one thing:	§14.4.4 vs §14.7 "rests on structure": scope the "one thing" claim (audit superlative B).
263	Chapter 14: Political Sustainability — Protecting Transformation Acros Step 2b · raw	mandatory statutory architecture, staggered effective dates, and federal agreements that carry their own obligations.	mandatory statutory architecture and staggered effective dates.	§14.7 — federal agreements can no longer be listed among the durable protections.
264	Chapter 14: Political Sustainability — Protecting Transformation Acros Step 2b · raw	These signal categories — a change in the governing party’s posture	Figure 14.1’s signals are instances of four broader categories — a change in the governing party’s posture	BEYOND VERMONT box: "These ... are the same four categories" but Fig 14.1 has eight signals and no enforcement-challenge row (audit Ch14 #2).
265	Chapter 14: Political Sustainability — Protecting Transformation Acros Step 2b · raw	financial assumptions — are the same four categories that would precede	financial assumptions — and those same four categories would precede	BEYOND VERMONT box: "These ... are the same four categories" but Fig 14.1 has eight signals and no enforcement-challenge row (audit Ch14 #2).
266	Conclusion — What Vermont Proves, and What Remains to Be Proven Step 2b · raw	need to treat January 2028 as an immovable deadline	need to treat October 2027 as an immovable deadline	--- Conclusion
267	Conclusion — What Vermont Proves, and What Remains to Be Proven Step 2b · raw	— April 2026	— July 2026	Signature: the Conclusion reports the July 2026 withdrawal, so it cannot be signed April 2026 (Ch13 opens "Updated July 2026").
268	Chapter 13: The Future of Healthcare Transformation — 2026, What Vermo Step 2b · raw	Five-pillars	Five-pillar	#ERROR!
269	Chapter 15: Healthcare Transformation as Portfolio Management — Applyi Step 2b · row_regex	in row “R-04 2026 governor committed to Act 68 modification; mandatory architecture at risk Policy Low–Med Critical 12 Accept + monitor: build irreversibility; document progress; legislative relat”: 12	12.5	Ch15 R-04: caption legend scores hyphenated bands at the midpoint: Low–Med 2.5 x Critical 5 = 12.5.
270	Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework fo Step 2b · row_regex	in row “White River Jct. Mt. Ascutney Hospital ~59K, aging, Dartmouth Health connection REH or focused-scope transition; cross-border coordination; rehabilitation COE”: White River Jct\.	Windsor	Ch16 Fig 16.3: Mt. Ascutney anchors the Windsor HSA (Upper Valley HSA merged into Windsor); App A and App C (as fixed) put the hospital in Windsor. DVHA Blueprint grant listing.
271	Appendix A — Vermont System Portrait Step 2b · raw	112 family medicine, 190 other primary-care specialties (RHT application)	Including 112 family medicine and 190 other primary-care specialties (RHT application)	App A: 370 FTE "112 family medicine, 190 other" sums to 302 (audit finding 5) — make the breakdown non-exhaustive rather than invent the missing 68.
272	Appendix A — Vermont System Portrait Step 2b · raw	shortfall by 2030 (112 family medicine, 190 other).	shortfall by 2030 (including 112 family medicine and 190 other).	App A: 370 FTE "112 family medicine, 190 other" sums to 302 (audit finding 5) — make the breakdown non-exhaustive rather than invent the missing 68.
273	Appendix B — Act 68 Statutory Timeline and Milestones Step 2b · raw	Vermont RHT: \$195M over 5 years for CIN and IT infrastructure	Vermont RHT: \$195M FY2026 award, with annual awards through FY2030, for CIN and IT infrastructure	App B Fig B.2: "\$195M over 5 years" contradicts every other statement (first-year award; per year through FY2030 — author-settled 2026-10-04; FY2026 award \$195,053,740).

274	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	This appendix describes the 19 HTR Research Lab tools referenced throughout the book.	This appendix describes 19 core HTR Research Lab tools referenced throughout the book.	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
275	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.1 FHIR Interoperability Lab	D.1 FHIR Interoperability Lab	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
276	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.2 Risk Stratification Engine	D.2 Risk Stratification Engine	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
277	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.3 APM Design Lab	D.3 APM Design Lab	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
278	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.4 APM Shared Savings Calculator	D.4 APM Shared Savings Calculator	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
279	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.5 Population Health Modeler	D.5 Population Health Modeler	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
280	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.6 Health Equity Studio	D.6 Health Equity Studio	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
281	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.7 Policy Simulator	D.7 Policy Simulator	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
282	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.8 Clinical Quality Optimizer	D.8 Clinical Quality Optimizer	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
283	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.9 Hospital Financial Scorecard	D.9 Hospital Financial Stress Test	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
284	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.10 Actuarial Lab	D.10 Actuarial Lab	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
285	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.11 HTA Studio	D.11 HTA Studio	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
286	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.12 AI Analytics Lab	D.12 AI Clinical Governance Lab	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).

287	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.13 Digital Health Lab	D.13 Digital Health Lab	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
288	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.14 Evidence Library	D.14 Evidence Library	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
289	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.15 Workforce Modeler	D.15 Workforce Modeler	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
290	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.16 Research Workspace	D.16 Research Workspace	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
291	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.17 Innovation Leaderboard	D.17 Innovation Leaderboard	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
292	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.18 VBC Transformation Readiness Assessment	D.18 VBC Transformation Readiness Assessment	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
293	Appendix D — HTR Platform Tools and Methodology Overview Step 2b · raw	E.19 The Wire	D.19 The Wire	App D: items numbered E.1–E.19 inside Appendix D (audit finding 16); two names do not match the platform registry (frontend/lib/taxonomy/tools.ts: "Hospital Financial Stress Test", "AI Clinical Governance Lab" — same descriptions).
294	Appendix E — Vermont Transformation Scorecard: 2022 Baseline, 2026 Sta Step 2b · raw	UVMMC 267% of benchmark; system avg 91% above	UVMMC 267% of benchmark; PPS avg 91% above	App E Fig E.6: the 91% premium is the PPS-group figure (\$2,730 vs \$1,427, Ch11 Fig 11.4).
295	Appendix F — A Reader’s Guide to the Health Transformation Review Plat Step 2b · row_regex	in row “Policy Regulation & legislation, public-health mandates, comparative policy, feasibility studies Chapter 1”: Chapter 1	Chapters 2–3	App F F.3: pillar->chapter map wrong on all five rows (audit finding 14); truth per frontend/lib/taxonomy/chapters.ts and the Heading1s.
296	Appendix F — A Reader’s Guide to the Health Transformation Review Plat Step 2b · row_regex	in row “Economics VBC models, market & finance, labor & workforce strategy, investment trends Chapters 8–9”: Chapters 8–9	Chapters 6–7	App F F.3: pillar->chapter map wrong on all five rows (audit finding 14); truth per frontend/lib/taxonomy/chapters.ts and the Heading1s.
297	Appendix F — A Reader’s Guide to the Health Transformation Review Plat Step 2b · row_regex	in row “Technology AI & machine learning, digital health & telemedicine, data security & governance, tech-enabled workflow Chapters 6–7”: Chapters 6–7	Chapters 4–5	App F F.3: pillar->chapter map wrong on all five rows (audit finding 14); truth per frontend/lib/taxonomy/chapters.ts and the Heading1s.
298	Appendix F — A Reader’s Guide to the Health Transformation Review Plat Step 2b · row_regex	in row “Clinical Hospital-at-home, precision medicine, virtual care models Chapters 10–11”: Chapters 10–11	Chapters 8–9	App F F.3: pillar->chapter map wrong on all five rows (audit finding 14); truth per frontend/lib/taxonomy/chapters.ts and the Heading1s.
299	Appendix F — A Reader’s Guide to the Health Transformation Review Plat Step 2b · row_regex	in row “Operations Revenue cycle, workforce & human capital, quality & compliance, supply chain, payer/network operations Chapters 14–15”: Chapters 14–15	Chapter 11	App F F.3: pillar->chapter map wrong on all five rows (audit finding 14); truth per frontend/lib/taxonomy/chapters.ts and the Heading1s.

300	Appendix F — A Reader’s Guide to the Health Transformation Review Plat Step 2b · raw	(Policy Simulator, Hospital Financial Scorecard, Actuarial Lab,	(Policy Simulator, Hospital Financial Stress Test, Actuarial Lab,	App F F.5 / F.8: registry names, and Appendix D does not hold "full specifications" for the four F.5 tools it omits.
301	Appendix F — A Reader’s Guide to the Health Transformation Review Plat Step 2b · raw	(HTA Studio, AI Analytics Lab, Digital Health Lab)	(HTA Studio, AI Clinical Governance Lab, Digital Health Lab)	App F F.5 / F.8: registry names, and Appendix D does not hold "full specifications" for the four F.5 tools it omits.
302	Appendix F — A Reader’s Guide to the Health Transformation Review Plat Step 2b · raw	Full tool specifications are in Appendix D.	Specifications for the 19 core tools are in Appendix D.	App F F.5 / F.8: registry names, and Appendix D does not hold "full specifications" for the four F.5 tools it omits.
303	Appendix F — A Reader’s Guide to the Health Transformation Review Plat Step 2b · raw	the VBC Readiness Assessment and Hospital Financial Scorecard,	the VBC Readiness Assessment and Hospital Financial Stress Test,	App F F.5 / F.8: registry names, and Appendix D does not hold "full specifications" for the four F.5 tools it omits.
304	Appendix H — The HTR Lab Workbook: A Five-Pillar Practicum Step 2b · para_regex	in paragraph “The Stage 5 question: Does the design penalize safety-net providers or widen disparities? Social risk adjustment belongs in the Stage 3 budget design — return there if the equity numbers fail.”: The Stage 5 question:	The equity question:	App H: H.6 is the Equity Imperative, not Stage 5 (audit finding 19).
305	Appendix H — The HTR Lab Workbook: A Five-Pillar Practicum Step 2b · raw	(Vermont’s \$1,303 per discharge)	(Vermont PPS hospitals’ \$1,303 per discharge above benchmark)	App H H.7: \$1,303 is the PPS-group gap, per Ch11 Fig 11.4.
306	Chapter 5: The Technology Pillar in Practice — FHIR, AI Governance, an Step 2c · raw	JAMA Network Open ambient scribe study (2024).	NEJM AI ambient scribe randomized trial (2025); JAMA ambient scribe study (2026).	C5-5 SOURCE LINE vs ITS OWN TABLE. Fig 5.3 now cites "(NEJM AI RCT, 2025; JAMA, 2026)" for the 15-25 min/day scribe figure (corrected 2026-10-04); the chapter Sources line still lists the 2024 JAMA Network Open study that carried the removed 30-45 min figure. Citation text mirrors the table's own.
307	Chapter 6: The Economics Pillar — Global Budgets, Reference-Based Pric Step 2c · raw	was established in 1972 with statutory authority	was established in 1971 with statutory authority	C6-6 WRONG FACT: HSCRC was created by 1971 legislation (rate-setting began July 1974) [HSCRC; MD State Archives].
308	Chapter 6: The Economics Pillar — Global Budgets, Reference-Based Pric Step 2c · raw	(both in Burlington)	(both in the Burlington area)	C6-10 IMPRECISE: Vermont's two ASCs are in Colchester (Green Mountain Surgery Center, opened 2019) and South Burlington (Vermont Eye Surgery Laser Center) (VTDigger 2019-03-22; Becker's ASC). Neither is in Burlington proper.
309	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · raw	This domain is covered in depth elsewhere in this book.	This domain is covered in depth in Chapter 11 (§11.11–11.12).	C7-8 DANGLING CROSS-REFERENCE (audit_ch07 Other E): "covered in depth elsewhere in this book". The coverage is Chapter 11 §11.11 "Revenue Cycle Transformation" and §11.12 "HCC Coding Excellence" (Heading2s verified).
310	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · raw	Vermont: 245 suicide/SH ED visits per 10,000	Vermont: 245.5 suicide/SH ED visits per 10,000	C7-10 NUMBER DRIFT: "245" vs "245.5" used in Ch8 (x3) and Ch11 (x2).
311	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · raw	Every MH-related hospitalization prevented at \$15,000 average saves 50x the cost of a CoCM BHCM monthly salary cost.	Every MH-related hospitalization prevented saves \$15,000 on average — a material offset against the cost of a CoCM behavioral health care manager.	C7-11 IMPLAUSIBLE ARITHMETIC (audit_ch07 #5): "\$15,000 ... saves 50x the cost of a CoCM BHCM monthly salary" implies a \$300 monthly salary. Drops the multiplier rather than inventing a salary figure.
312	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 2c · raw	Total score below 60/120 indicates pre-transition status;	An overall readiness score below 60% indicates the organization is not yet ready for full-risk VBC;	C7-15 INTERNAL CONTRADICTION (audit_ch07 #7/#11): §7.4.2 now uses the tool's 0-4 scale and percentage bands (80% / 60%, matching VBCReadinessAssessment.tsx lines 264-265); Key Concepts still says "below 60/120 ... pre-transition".

313	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	65+ population growing 57% by 2040, exceeding 30% of total population	65+ population growing 57% by 2040, toward 30% of total population	C8-20 CONFLICT WITH PREFACE/INTRODUCTION (chapter yields): Preface "growing toward 30%", Introduction "rose from 21.7% toward 30%"; Ch8 says "exceeding 30%".
314	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	Dementia affects an estimated 14 million Americans nationally by 2060	Dementia is projected to affect an estimated 14 million Americans nationally by 2060	C8-21 GARBLED TENSE (audit_ch08 Other): a condition does not "affect ... by 2060". CDC projects ADRD will reach ~14 million Americans by 2060.
315	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	HRSA recognizes no primary care Health Professional Shortage Areas in Vermont.	Only a handful of Vermont areas meet HRSA's geographic criteria for a primary care Health Professional Shortage Area.	C8-28 WRONG FACT (audit_ch08 MISMATCH 5, flagged verify-externally): HRSA does designate primary care HPSAs in Vermont -- every FQHC/RHC site carries a facility HPSA and a few RSAs meet geographic criteria [VDH; UVM 2024 workforce brief]. Keeps Oliver Wyman's point (headcount is not the core problem). Same unqualified claim survives in Ch2, Ch11, App A (rule 13 -- see REPORT).
316	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	Per-member-per-month (PMPM) payments from all payers — Medicare, Medicaid, and commercial insurers — with a base payment	Per-member-per-month (PMPM) payments from Medicaid and commercial insurers — and from Medicare until the All-Payer ACO Model agreement ended in December 2025 — with a base payment	C8-32 STALE: Fig 8.1 says Blueprint PMPMs come "from all payers -- Medicare, Medicaid, and commercial". Medicare stopped paying in Dec 2025 [AHS25; VTPUB].
317	Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Trans Step 2c · raw	not just a promising approach, but the only integrated care model with a clear evidence base across 90+ randomized controlled trials, and the only model with dedicated Medicare billing codes (CPT codes 99492, 99493, 99494).	not just a promising approach, given the evidence base and Medicare billing codes described above.	C8-33 REPETITION (audit_ch08 "Genuine repetition"): §8.3.2's opener restates, at length, the two CoCM facts stated in the §8.3 stat block and §8.3.1 Layer 1.
318	Appendix B — Act 68 Statutory Timeline and Milestones Step 2e · raw	Status (April 2026)	Status (April 2026; July 2026 updates)	Appendix B header: rows now carry July 2026 updates (steps 1, 2d); header said April 2026 only.
319	Chapter 1: The Five-Pillar Framework and the Execution Sequence Step 3 · raw	Three sequencing gaps compounding over the federal agreement's ~8-year run	Three sequencing gaps compounding over the federal agreement's 2017–2025 run	C1-1 CONTRADICTION (low). Fig 1.1 '~8-year run' vs Introduction 'over nine years' and Ch2 §2.7.2 'from 2017 to 2025'. Dating the run removes the count conflict without editing the Introduction (the chapter yields). Anchor count: 1.
320	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 3 · raw	HRSA recognizes no Health Profession Shortage Areas in Vermont.	Only a handful of Vermont areas meet HRSA's geographic criteria for a primary care Health Professional Shortage Area.	C2-7 WRONG FACT (medium). Fig 2.1. Identical replacement to the committed Ch8 fix C8-28. RULE 13: the same unqualified claim survives OUTSIDE this unit at Ch11 (md ~5039) and Appendix A (md ~6704). Sources: https://healthvermont.gov/systems/health-professionals/shortages-and-designations ; https://uvm.edu/d10-files/documents/2024-06/Addressing-the-Shortage-of-H-ealthcare-Workers.pdf Anchor count: 1.
321	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 3 · raw	Under realistic assumptions (7-8% expense growth)	Under realistic assumptions (10% labor, 7-8% other expense growth)	C2-8 CONTRADICTION (low). Callout mislabels the scenario defined two paragraphs earlier (10% non-physician labor, 7-8% other). Prior audit M14, still present. Anchor count: 1.
322	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 3 · raw	Hospital charges grew 38% since 2018; median household income grew only 22%	Hospital charges grew 38% from 2018 to 2022; median household income grew only 22% over the same period	C2-9 CONTRADICTION (low). Callout widens the 2018–2022 window stated in §2.3.1. Prior audit M15, still present. Anchor count: 1.
323	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 3 · raw	By the mid-2030s, Vermont will have nearly as many residents over 65 as under 20.	As the Preface notes, Vermont already has more residents over 65 than under 18.	C2-10 WRONG FACT (medium). Census Vintage 2024: Vermont is one of 11 states with more 65+ residents than children (~1.3 per child). The Preface box already states it, so the chapter cross-references it instead of predicting a crossover that has happened. Prior audit M16. Sources: https://www.yahoo.com/news/older-adults-now-outnumber-children-161103511.html (AP on Census Vintage 2024 estimates); Preface BEYOND VERMONT box (book) Anchor count: 1.

324	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	Episode-based payment for acute and post-acute care episodes. Voluntary. For health systems and post-acute providers, BPCI-A remains a viable two-sided risk VBC entry point for specific service lines (joint replacement, cardiac, etc.).	Episode-based payment for acute and post-acute care episodes. Voluntary. Ended December 31, 2025; CMS's mandatory Transforming Episode Accountability Model (TEAM, 2026–2030) now carries episode-based risk for selected surgical episodes.	C3-3 STALE (high). Fig 3.2 calls an ended model 'viable'. Fig 3.1's 'without' mentions of BPCI are historical and stay. Sources: https://www.milliman.com/en/insight/next-generation-medicare-bundled-payments-considerations-team ; https://www.medbridge.com/blog/team-model-from-cms-a-strategic-shift-toward-surgical-episode-accountability Anchor count: 1.
325	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	ACO REACH is the highest-accountability current federal ACO option.	ACO REACH is the highest-accountability federal ACO option through its final performance year, 2026; CMS's voluntary LEAD model succeeds it in January 2027.	C3-4 STALE (medium). Fig 3.2; REACH ends 31 Dec 2026 (LEAD announced 18 Dec 2025, runs 2027–2036). Sources: https://www.healthcarediver.com/news/aco-lead-medicare-accountable-cms-reach-replacement/808356/ ; https://www.techtarget.com/revcyclemanagement/news/366636685/CMS-announces-new-ACO-model-as-REACH-ends Anchor count: 1.
326	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	Medicaid launch May 2026; Medicare Part D January 2028.	Medicaid launch May 2026; Medicare Part D launch postponed by CMS in April 2026, with a temporary Medicare GLP-1 Bridge demonstration in its place.	C3-5 WRONG FACT (medium). Fig 3.2 BALANCE row. Part D was announced for Jan 2027, then postponed on 21 Apr 2026; no 2028 date has been announced. Sources: https://kff.org/medicare/what-to-know-about-the-balance-model-for-glp-1s-in-medicare-and-medicaid ; https://www.fiercehealthcare.com/regulatory/cms-launching-payment-model-boost-access-glp-1s-medicaid-part-d Anchor count: 1.
327	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	Late 2026 (states must implement by December 31, 2026)	January 1, 2027	C3-8 STALE (medium). Fig 3.4 timing cell; fixed fact #4 (the §3.6 bullet just above already says January 1, 2027). Sources: https://vtlawhelp.org/2027-medicaid-changes ; https://dvha.vermont.gov/medicaid-work-requirements-screening-tool Anchor count: 1.
328	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	H.R. 1 (2025) included additional prior authorization reform provisions: public reporting of PA denial rates by service type, required disclosure of clinical criteria used in PA decisions, and stronger external appeal rights for denied requests.	H.R. 1 (2025) did not include prior authorization reform; proposals for public reporting of PA denial rates by service type, required disclosure of clinical criteria used in PA decisions, and stronger external appeal rights for denied requests remain pending legislation, not law.	C3-11 WRONG FACT (high). §3.4.1. OBBBA has no PA provisions (K&S provision-by- provision review; Dec 2025 coalition still urging Congress to pass PA reform). The next sentence ('These are incremental improvements...') still reads correctly. Sources: https://www.kslaw.com/insights/articles/the-one-big-beautiful-bill-act-explained-a-detailed-review-of-key-changes-for-the-healthcare-industry ; https://www.asam.org/news/detail/2025/12/09/in-coalition--asam-urges-congress-to-pass-prior-authorization-reforms Anchor count: 1.
329	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	What H.R. 1 and CMS Rules Require	What CMS Rules Require	C3-12 WRONG FACT (medium). §3.4.1 heading title run (the number is a separate run): becomes 'Federal Prior Authorization Reform: What CMS Rules Require'. Not a recurring style-critical heading. Sources: https://www.kslaw.com/insights/articles/the-one-big-beautiful-bill-act-explained-a-detailed-review-of-key-changes-for-the-healthcare-industry ; https://www.asam.org/news/detail/2025/12/09/in-coalition--asam-urges-congress-to-pass-prior-authorization-reforms Anchor count: 1.
330	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	directing AHS to develop simplified prior authorization processes and directing GMCB to report on prior authorization burden and denial rates	directing the Department of Financial Regulation to analyze insurers' prior authorization data and, with GMCB, to recommend statutory changes to align and streamline prior authorization across insurers	C3-13 WRONG FACT (medium). Act 167 Sec. 11 assigns PA to DFR (with GMCB), not AHS. Sources: https://legislature.vermont.gov/Documents/2022/Docs/ACTS/ACT167/ACT167%20As%20Enacted.pdf ; https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20As%20Enacted.pdf Anchor count: 1.
331	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	Act 68 builds on this by requiring insurers to demonstrate that administrative cost structures, including PA programs, are consistent with medical loss ratio requirements.		C3-14 WRONG FACT (medium). Act 68 as enacted contains no prior-authorization or medical-loss-ratio provision (full text searched). Sources: https://legislature.vermont.gov/Documents/2022/Docs/ACTS/ACT167/ACT167%20As%20Enacted.pdf ; https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20As%20Enacted.pdf Anchor count: 1.

332	Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strateg Step 3 · raw	Act 167 and Act 68 both address PA simplification.	Act 167 directed state work on PA simplification.	C3-15 WRONG FACT (low). Key Concepts; consequence of C3-14. Sources: https://legislature.vermont.gov/Documents/2022/Docs/ACTS/ACT167/ACT167%20As%20Enacted.pdf ; https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20As%20Enacted.pdf Anchor count: 1.
333	Chapter 4: The Technology Pillar — Data Infrastructure for a Transform Step 3 · raw	AHS HSA Coordinator model (Chapter 3)	AHS HSA Coordinator model (Chapter 16)	C4-2 DANGLING REF (low). Fig 4.3: the HSA Coordinator model is developed in Ch16 ('The mechanism is the HSA Coordinator'); Ch3 mentions the positions once. Anchor count: 1.
334	Chapter 4: The Technology Pillar — Data Infrastructure for a Transform Step 3 · raw	recovering 30 to 45 minutes per provider per day	recovering 15 to 25 minutes per provider per day	C4-3 CONTRADICTION (medium). §4.6.1 keeps the 30-45 min figure removed from Ch5. The Ch4 Sources line still lists the 2024 JAMA Netw Open study; it supports the qualitative sentence and is left. Sources: Ch5 Fig 5.6 (book, corrected 2026-10-04): 'ambient scribes save ~15–25 min/day/provider (NEJM AI RCT, 2025; JAMA, 2026)' Anchor count: 1.
335	Chapter 4: The Technology Pillar — Data Infrastructure for a Transform Step 3 · raw	among the 47 state applications submitted in late 2025	among the 50 state applications submitted in late 2025	C4-4 WRONG FACT (medium). §4.6: all 50 states applied and all received awards (29 Dec 2025). Sources: https://www.aamc.org/advocacy-policy/washington-highlights/cms-awards-rhtp-funding-all-50-states ; https://nasmhpd.org/all-50-states-apply-for-rural-health-transformation-program Anchor count: 1.
336	Chapter 4: The Technology Pillar — Data Infrastructure for a Transform Step 3 · raw	the vehicle through which the Act 68 §10 integration mandate is actually delivered	the vehicle Act 68 §10 directs AHS to develop for that integration	C4-9 WRONG FACT (medium). Act 68 §10 (18 V.S.A. § 9353(b)(4)) makes integration conditional: AHS must 'determine whether to integrate'; integration may not begin before 1 Jan 2027 and only on a majority vote of the HIE Steering Committee. Sources: https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20As%20Enacted.pdf ; https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20Act%20Summary.pdf Anchor count: 1.
337	Chapter 4: The Technology Pillar — Data Infrastructure for a Transform Step 3 · raw	§10 legislatively mandated the integration of clinical, claims, and SDOH data into a single record, and required AHS	§10 directed AHS to develop the UHDS and to decide whether to integrate clinical, claims, and SDOH data into a single record — integration may not begin before January 1, 2027, and only on a majority vote of the HIE Steering Committee — and required AHS	C4-10 WRONG FACT (medium). §4.3.5; see C4-9. The Jan 15, 2026 report deadline in the same sentence is correct and stays. Sources: https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20As%20Enacted.pdf ; https://legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20Act%20Summary.pdf Anchor count: 1.
338	Chapter 9: The Clinical Pillar in Practice — Care Model Implementation Step 3 · raw	without specifying complication status is leaving RAF value uncaptured	without documenting the complications it has caused — diabetic CKD, for example, carries its own HCC — is leaving RAF value uncaptured	C9-14 WRONG FACT (stale risk model). Under CMS-HCC V28 (100% in PY2026) all diabetes HCCs carry one coefficient, so the complication status of the diabetes code no longer moves RAF; separately-mapped complication HCCs (e.g. CKD) do [V28].
339	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	shows clear disparities: Black adults in Vermont have an 82% health insurance coverage rate compared to 94% of Vermont adults statewide (BRFSS self-report). Adults who are Asian, Native Hawaiian or Pacific Islander, Black, or another race are significantly less likely to have a primary care provider than white adults. LGBTQ+ Vermonters, Vermonters with disabilities, and lower-income Vermonters delay care at higher rates due to cost.	shows clear disparities in coverage, primary care access, and cost-related delays in care, detailed below.	C10-1 REPETITION. §10.1 restates §10.2.2's 94%/82% pair, PCP gap and delay-care list (audit_ch10 repetition 1-2). The "(BRFSS self-report)" label moves to §10.2.2's first use so the base stays labelled.

340	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	statewide, 94% of Vermont adults have health insurance — but only 82% of Black adults.	statewide, 94% of Vermont adults have health insurance (BRFSS self-report) — but only 82% of Black adults.	C10-1 REPETITION. §10.1 restates §10.2.2's 94%/82% pair, PCP gap and delay-care list (audit_ch10 repetition 1-2). The "(BRFSS self-report)" label moves to §10.2.2's first use so the base stays labelled.
341	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	concentrates the state's most severe access, affordability, and outcome disparities:	concentrates the state's most severe coverage and poverty disparities, while behavioral health crisis burdens concentrate elsewhere:	C10-2 CONTRADICTION (audit_ch10 MISMATCH 1). Fig 10.1 directly below shows only coverage and poverty as NEK cells; both behavioral-health cells name other counties (Rutland/Windham; Chittenden/Bennington/Windham).
342	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	North Country and Northeastern Vermont Regional are candidates for Tier 3 designation under the Oliver Wyman regionalization blueprint — meaning they may not be able to sustain	North Country is among the Tier 3 hospitals whose future the Oliver Wyman regionalization blueprint leaves for further assessment, while Northeastern Vermont Regional is assigned a focused (Tier 2) scope — meaning North Country may not be able to sustain	C10-3 CONTRADICTION. Ch11 Fig 11.5 / §11.5.1 and Appendix C assign NVRH to Tier 2 (4 COEs) and North Country to Tier 3; Ch10 calls both Tier 3 candidates.
343	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	down from 16 in late 2025, as four states eliminated coverage in response to budget pressure from Medicaid cuts.	down from 16 in October 2025: four states eliminated coverage in response to budget pressure, while North Carolina reinstated coverage it had briefly dropped.	C10-8 WRONG FACT / arithmetic (audit_ch10 MISMATCH 4): 16 - 4 != 13 [KFFGLP].
344	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	Vermont Medicaid covers GLP-1s for type 2 diabetes management but not for obesity as a primary indication.		C10-9 REPETITION: back-to-back sentences stating the same coverage rule.
345	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	and in Medicare Part D in January 2028, with	and in Medicare Part D no earlier than 2028 (CMS postponed its planned January 2027 start), with	C10-10 / C10-11 STALE: Medicare BALANCE was set for Jan 2027, then postponed; 2028 is possible, not scheduled [KFFBAL][BALRFA].
346	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	Medicare Part D in January 2028. Voluntary	Medicare Part D postponed from January 2027, no earlier than 2028. Voluntary	C10-10 / C10-11 STALE: Medicare BALANCE was set for Jan 2027, then postponed; 2028 is possible, not scheduled [KFFBAL][BALRFA].
347	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	Decision needed by January 2026 (BALANCE opt-in deadline); coverage effective May 2026 if Vermont participates	State applications due July 31, 2026 (BALANCE RFA); coverage starts between May 2026 and January 1, 2027 if Vermont participates	C10-12 CONTRADICTION + WRONG DATE (audit_ch10 MISMATCH 5): Fig 10.4 says Jan 2026, Fig 10.6 says June 2026. Real state deadline July 31, 2026; agreements executed May 1, 2026 - Jan 1, 2027 [BALRFA][KFFBAL]. Vermont's actual decision could not be verified, so both cells stay conditional.
348	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	BALANCE Model participation decision by June 2026	BALANCE Model participation decision by July 31, 2026 (state application deadline)	C10-12 CONTRADICTION + WRONG DATE (audit_ch10 MISMATCH 5): Fig 10.4 says Jan 2026, Fig 10.6 says June 2026. Real state deadline July 31, 2026; agreements executed May 1, 2026 - Jan 1, 2027 [BALRFA][KFFBAL]. Vermont's actual decision could not be verified, so both cells stay conditional.
349	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	Vermont HEROI: 14 HSA-level equity scores; GMCB reporting requirement	HEROI: 14 HSA-level equity scores once computed; candidate GMCB reporting metric	C10-19 CONTRADICTION (audit_ch10 MISMATCH 8). §10.13 defines HEROI as the Health Equity Studio's proprietary framework and says Vermont's score "has not been formally computed"; HEROI is the book's own coined term (memory project_queue_execution #8). Three passages present it as a GMCB requirement, a Vermont policy innovation, and a statutory requirement.
350	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	Vermont's HEROI scoring framework is a policy innovation worth protecting. It is the only state-level tool that systematically scores	HEROI scoring is a framework Vermont should adopt and protect. It systematically scores	C10-19 CONTRADICTION (audit_ch10 MISMATCH 8). §10.13 defines HEROI as the Health Equity Studio's proprietary framework and says Vermont's score "has not been formally computed"; HEROI is the book's own coined term (memory project_queue_execution #8). Three passages present it as a GMCB requirement, a Vermont policy innovation, and a statutory requirement.

351	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	the HEROI framework, the Health Equity Advisory Commission	the Health Equity Advisory Commission	C10-19 CONTRADICTION (audit_ch10 MISMATCH 8). §10.13 defines HEROI as the Health Equity Studio's proprietary framework and says Vermont's score "has not been formally computed"; HEROI is the book's own coined term (memory project_queue_execution #8). Three passages present it as a GMCB requirement, a Vermont policy innovation, and a statutory requirement.
352	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 3 · raw	Follow-up after ED visit for mental illness (FUH); follow-up after ED visit for alcohol use (FUA)	Follow-up after ED visit for mental illness (FUM); follow-up after ED visit for substance use (FUA)	C10-20 WRONG FACT: Fig 10.7 HEDIS acronyms [HEDIS]. Ch9 Key Concepts already uses FUM correctly for the ED-visit measure.
353	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	It has a newly contracted operational partner in the Rural Health Redesign Center.	It had a contracted operational partner in the Rural Health Redesign Center through October 2025.	C11-2 CONTRADICTION: §11.3 and Key Concepts say the RHRC contract concluded October 2025.
354	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	Mt. Ascutney (White River Junction)	Mt. Ascutney (Windsor)	C11-5 WRONG FACT x2: Mt. Ascutney is in Windsor [MAH]; Appendix C / Ch16 fixed the same way by other step-3 ops.
355	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	Mt. Ascutney (White River Jct.)	Mt. Ascutney (Windsor)	C11-5 WRONG FACT x2: Mt. Ascutney is in Windsor [MAH]; Appendix C / Ch16 fixed the same way by other step-3 ops.
356	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	a headcount problem: HRSA recognizes no Health Profession Shortage Areas in Vermont, and if	a headcount problem: only a handful of Vermont areas meet HRSA's geographic criteria for a primary care Health Professional Shortage Area, and if	C11-6 WRONG FACT [HPSA]; wording mirrors the Ch8 C8-28 fix. Keeps Oliver Wyman's point that headcount is not the core problem.
357	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	370 FTEs 112 family medicine, 190 other primary care	370 FTEs including 112 family medicine, 190 other primary care	C11-7 ARITHMETIC (audit_ch11 MISMATCH 6): 112 + 190 = 302, not 370. Appendix A already says "including"; the remaining 68 FTE breakdown was not verified.
358	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	‘diabetes’ (HCC 19, lower RAF) versus ‘diabetes with diabetic CKD’ (HCC 18 + HCC 136, higher RAF)	‘diabetes’ alone (one diabetes HCC) versus ‘diabetes with diabetic CKD’ (the diabetes HCC plus a separate CKD HCC, higher RAF)	C11-14 WRONG FACT (stale risk model): V24 HCC numbers 18/19/136 [V28].
359	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 3 · raw	working with all 14 hospitals on transformation planning — is the mechanism	working with all 14 hospitals on transformation planning — was the mechanism	C11-15 STALE tense: next sentence says the RHRC contract concluded Oct 2025.
360	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 3 · raw	The infrastructure described in this chapter	The infrastructure this chapter calls for	C12-9 DANGLING REF (audit_ch12 MISMATCH 2). The body never describes a PMO, learning collaboratives or Communities of Practice ("learning collaborative" occurs book-wide only in these frame sentences); the PMO is developed in Ch15.
361	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 3 · raw	The knowledge transfer infrastructure this chapter describes	The knowledge transfer infrastructure this chapter calls for	C12-9 DANGLING REF (audit_ch12 MISMATCH 2). The body never describes a PMO, learning collaboratives or Communities of Practice ("learning collaborative" occurs book-wide only in these frame sentences); the PMO is developed in Ch15.
362	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 3 · raw	The Transformation PMO this chapter describes	The Transformation PMO described in Chapter 15	C12-9 DANGLING REF (audit_ch12 MISMATCH 2). The body never describes a PMO, learning collaboratives or Communities of Practice ("learning collaborative" occurs book-wide only in these frame sentences); the PMO is developed in Ch15.
363	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 3 · raw	This chapter describes infrastructure that is necessary	This chapter calls for infrastructure that is necessary	C12-9 DANGLING REF (audit_ch12 MISMATCH 2). The body never describes a PMO, learning collaboratives or Communities of Practice ("learning collaborative" occurs book-wide only in these frame sentences); the PMO is developed in Ch15.

364	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 3 · raw	Weekly synthesis of the most consequential	Continuously updated synthesis, with a daily briefing, of the most consequential	C12-8 CONTRADICTION: three cadences for The Wire. Platform (frontend/app/the-wire: "updated continuously", "Daily Briefing") and the Introduction ("a daily intelligence feed") win; the weekly product is the Convergence Newsletter, which §12.2.1 already names.
365	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 3 · raw	The weekly policy and practice intelligence service	The continuously updated policy and practice intelligence feed, with a daily briefing	C12-8 CONTRADICTION: three cadences for The Wire. Platform (frontend/app/the-wire: "updated continuously", "Daily Briefing") and the Introduction ("a daily intelligence feed") win; the weekly product is the Convergence Newsletter, which §12.2.1 already names.
366	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 3 · raw	reporting losses, \$2.4B 5-year deficit	reporting losses, \$700M–\$2.4B 5-year deficit	C12-7 FRONT-MATTER CONFLICT: the Preface gives the deficit as a \$700M-\$2.4B range.
367	Chapter 12: Infrastructure for Knowledge Transfer and Implementation Step 3 · raw	The Wire	The Wire (formatting/structure change)	
368	Appendix A — Vermont System Portrait Step 3c · raw	HRSA recognizes no Health Profession Shortage Areas in Vermont; the problem	only a handful of Vermont areas meet HRSA's geographic criteria for a primary care Health Professional Shortage Area; the problem	C2-7 WRONG FACT (medium). Fig 2.1. Identical replacement to the committed Ch8 fix C8-28. RULE 13: the same unqualified claim survives OUTSIDE this unit at Ch11 (md ~5039) and Appendix A (md ~6704). Sources: https://healthvermont.gov/systems/health-professionals/shortages-and-designations ; https://uvm.edu/d10-files/documents/2024-06/Addressing-the-Shortage-of-Healthcare-Workers.pdf Anchor count: 1.
369	Appendix B — Act 68 Statutory Timeline and Milestones Step 3c · row_regex	in row “January 2028 BALANCE Model Medicare Part D coverage launches CMS Federal program”: January 2028	Postponed	Phase 1, step 3c — rule-13 sweep: errors the step-3 analysts found that sit outside their units. - Appendix A: "HRSA recognizes no Health Profession Shortage Areas in Vermont" — same wording fix as Ch2/Ch8/Ch11 (HRSA designates a handful of Vermont primary-care HPSAs). - Appendix B: BALANCE Medicare Part D launch — CMS announced 21 Apr 2026 it will not launch in Part D as planned; no new date (AHA 2026-04-22; KFF "What to Know About the BALANCE Model"). - Ch7 §7.8 heading (body + TOC): "Making Care Primary" — MCP ended early 31 Dec 2025 and Vermont was never in it; the section body is about primary-care investment, not MCP.
370	Appendix B — Act 68 Statutory Timeline and Milestones Step 3c · raw	BALANCE Model Medicare Part D coverage launches	BALANCE Model Medicare Part D coverage (planned 2027; postponed by CMS in April 2026, no new date)	Phase 1, step 3c — rule-13 sweep: errors the step-3 analysts found that sit outside their units. - Appendix A: "HRSA recognizes no Health Profession Shortage Areas in Vermont" — same wording fix as Ch2/Ch8/Ch11 (HRSA designates a handful of Vermont primary-care HPSAs). - Appendix B: BALANCE Medicare Part D launch — CMS announced 21 Apr 2026 it will not launch in Part D as planned; no new date (AHA 2026-04-22; KFF "What to Know About the BALANCE Model"). - Ch7 §7.8 heading (body + TOC): "Making Care Primary" — MCP ended early 31 Dec 2025 and Vermont was never in it; the section body is about primary-care investment, not MCP.
371	(front matter) Step 3c · raw	7.8 Making Care Primary — Vermont’s Primary Care Economics	7.8 Primary Care Investment — Vermont’s Primary Care Economics	Phase 1, step 3c — rule-13 sweep: errors the step-3 analysts found that sit outside their units. - Appendix A: "HRSA recognizes no Health Profession Shortage Areas in Vermont" — same wording fix as Ch2/Ch8/Ch11 (HRSA designates a handful of Vermont primary-care HPSAs). - Appendix B: BALANCE Medicare Part D launch — CMS announced 21 Apr 2026 it will not launch in Part D as planned; no new date (AHA 2026-04-22; KFF "What to Know About the BALANCE Model"). - Ch7 §7.8 heading (body + TOC): "Making Care Primary" — MCP ended early 31 Dec 2025 and Vermont was never in it; the section body is about primary-care investment, not MCP.

372	Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling a Step 3c · raw	Making Care Primary — Vermont's Primary Care Economics	Primary Care Investment — Vermont's Primary Care Economics	Phase 1, step 3c — rule-13 sweep: errors the step-3 analysts found that sit outside their units. - Appendix A: "HRSA recognizes no Health Profession Shortage Areas in Vermont" — same wording fix as Ch2/Ch8/Ch11 (HRSA designates a handful of Vermont primary-care HPSAs). - Appendix B: BALANCE Medicare Part D launch — CMS announced 21 Apr 2026 it will not launch in Part D as planned; no new date (AHA 2026-04-22; KFF "What to Know About the BALANCE Model"). - Ch7 §7.8 heading (body + TOC): "Making Care Primary" — MCP ended early 31 Dec 2025 and Vermont was never in it; the section body is about primary-care investment, not MCP.
373	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 5 · raw	Grace Cottage, Gifford Medical Center, North Country Hospital, and Porter Medical Center are among the facilities whose futures require further discussion as part of Vermont's regionalization planning.	Grace Cottage, Gifford Medical Center, and North Country Hospital are among the facilities whose futures require further discussion as part of Vermont's regionalization planning. Porter Medical Center's centers-of-excellence designations are also still under discussion, but it retains inpatient care as a Tier 2 facility within the UVM Health Network.	S5-1a Ch11 §11.5 Tier-3 paragraph: drop Porter from the Tier-3 list, keep the (true) COE-under-discussion point, place it in Tier 2.
374	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 5 · raw	Grace Cottage, Gifford, North Country, Porter	Grace Cottage, Gifford, North Country	S5-1b Fig 11.5 Tier-3 group row: remove Porter. The cell text alone occurs twice (Ch2 Fig 2.2 has the same string and is correct), so the anchor includes this row's unique w14:parald="00000E2B".
375	Chapter 11: The Operations Pillar — Executing Hospital System Transfor Step 5 · raw	Rehabilitation COE. Dartmouth Health affiliation creates cross-border care opportunities.	Porter Medical Center (Middlebury)TBDTier 2 — Focused scopeUVM Health Network affiliate; COE designations still under discussion. Oliver Wyman's long-term recommendation is to grow surgical services; interim options include converting inpatient beds to mental-health, geri-psych, or memory care.	S5-1c Fig 11.5: insert a Porter row (Tier 2) immediately after the Mt. Ascutney row, i.e. last of the Tier-2 block, before the Tier-3 row. Anchor = the unique Mt. Ascutney rationale text + the row close.
376	Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework fo Step 5 · raw	TBD tier; UVM-network affiliation; shared-service opportunities with UVMHC	Tier 2 (COE designations TBD); UVM-network affiliation; shared-service opportunities with UVMHC	S5-1d Ch16 HSA table, Middlebury row: "TBD tier" -> Tier 2 (COEs TBD).
377	PREFACE Step 5 · raw	Premiums had risen 108% in six years while household income grew 22%.	Premiums had risen 108% in six years; median household income grew 22% in the four years to 2022.	S5-3a Preface: decouple the periods.
378	INTRODUCTION Step 5 · raw	Median household income had grown 22% over the same period.	Median household income had grown 22% between 2018 and 2022.	S5-3b Introduction: "over the same period" (= 2018-2024) is wrong.
379	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 5 · raw	Hospital approved charge increases grew 38% over the same period, compared to income growth of 22%. The gap is structural: hospital price growth has been running at two to three times income growth,	Hospital approved charge increases compounded to 38% from FY2018 to FY2024. The gap is structural: premium growth has been running at two to three times the annual pace of income growth,	S5-3c + S5-3d Ch2 §2.3 prose: 38% is FY2018-FY2024 (not "the same period" as 2018-2022); and the "two to three times" ratio belongs to premiums.
380	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 5 · raw	Hospital charges grew 38% from 2018 to 2022; median household income grew only 22% over the same period	Hospital charges grew 38% from FY2018 to FY2024; median household income grew 22% from 2018 to 2022	S5-3e Ch2 box bullet: same two period errors.
381	Appendix A — Vermont System Portrait Step 5 · raw	a 108% increase — while median household income grew 22%, and out-of-pocket	a 108% increase — while median household income grew 22% (2018–2022), and out-of-pocket	S5-3f Appendix A (Vermont System Portrait): same implied same-period pairing. Found by the rule-13 grep; not in the original finding.

382	Chapter 2: The Policy Pillar — Legislative Architecture for Structural Step 5 · raw	50 public HSA-level community meetings averaging 68 participants each,	50 public HSA-level community meetings with an estimated 1,947 attendees in all (roughly 39 each),	
383	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 5 · tbl_regex	in row “Uninsured rate, Essex County (NE Kingdom) 8% vs. 3% statewide — nearly 3x the state average despite Vermont’s near-universal coverage Counties with above-average suicide/self-harm ED rates Rutland +”:	(formatting change only)	
384	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 5 · tbl_regex	in row “Uninsured rate, Essex County (NE Kingdom) 8% vs. 3% statewide — nearly 3x the state average despite Vermont’s near-universal coverage Counties with above-average suicide/self-harm ED rates Rutland +”:	(formatting change only)	
385	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 5 · tbl_regex	in row “Uninsured rate, Essex County (NE Kingdom) 8% vs. 3% statewide — nearly 3x the state average despite Vermont’s near-universal coverage Counties with above-average suicide/self-harm ED rates Rutland +”:	(formatting change only)	
386	Chapter 10: The Equity Imperative — Closing Gaps, Not Just Averaging T Step 5 · tbl_regex	Uninsured rate, Essex County (NE Kingdom) 8% vs. 3% statewide — nearly 3x the state average despite Vermont’s near-universal coverage Counties with above-average suicide/self-harm ED rates Rutland + Windham Statistically higher than state average per 10,000 ED visits (AHS Nov 2025 data)	(table row deleted)	
Copy decisions				